1 **Tuesday, 18 March 2025.**
2 **(10.00 am)**
3 **LADY HALLETT:** Mr Stoate.
4 **MR STOATE:** Good morning, my Lady. The first witness today,
5 please, is Daniel Mortimer. Can he please be sworn.
6 **MR DANIEL MORTIMER (sworn)**
7 **Questions from COUNSEL TO THE INQUIRY**
8 **MR STOATE:** Thank you. Could you give the court your full
9 name, please.
10 **A.** My name is Daniel Joseph Mortimer.
11 **Q.** Thank you. You very helpfully provided a witness
12 statement to the Inquiry, the reference for which is
13 INQ000513763. It's 54 pages long, and dated
14 21 November 2024. Is that statement true to the best of
15 your knowledge and belief?
16 **A.** It is.
17 **Q.** Thank you very much. By way of background, you are the
18 deputy chief executive of the NHS Confederation and the
19 CEO of NHS Employers, which is part of the
20 confederation; is that right?
21 **A.** That is correct.
22 **Q.** You held that position through the pandemic, and in fact
23 were the interim CEO of NHS Confederation between
24 October 2020 and June of 2021; is that right?
25 **A.** That is right, yes.

1 **Q.** Thank you. And as CEO of NHS Employers, you say you
2 have an understanding of how Covid-19 impacted the NHS
3 workforce. Can I ask you, please, just by way of brief
4 background, what is the NHS Confederation and what is
5 NHS Employers?
6 **A.** Of course.
7 The NHS Confederation is a membership organisation.
8 It represents the whole range of organisations that
9 commission and deliver healthcare in England, but also
10 in Wales and Northern Ireland. It's a charity and its
11 role as a membership organisation is to support our
12 members but also to act as their advocates and their
13 representatives, particularly with government, but also
14 with the public.
15 The confederation also delivers some services on
16 behalf of the Department of Health and Social Care and
17 other arm's-length bodies as well. NHS Employers is the
18 largest of those, there are others as well.
19 And NHS Employers is the employers organisation for
20 the NHS in England. It has a particular role in
21 representing what is the largest employer in the country
22 to government but it also delivers some services on
23 behalf of the Secretary of State, and the wider NHS, and
24 we have a particular responsibility for supporting the
25 management of the relationship with trade unions, in its

1 various forms.
2 **Q.** Thank you. You say at one point in your statement that
3 the members you represent employ some 1.5 million staff,
4 cared for more than a million patients a day and control
5 150 billion of public expenditure each year?
6 **A.** Yes.
7 **Q.** So a significant voice forum within the NHS; is that
8 a fair summary?
9 **A.** We aim to be so, yes.
10 **Q.** Thank you. In terms of your work, the work of the
11 NHS Confederation during the pandemic, did this include
12 collecting insights from members about the reality of
13 the ground?
14 **A.** Yes. Our particular role, or one of our particular
15 roles in the pandemic, was to collect those insights but
16 also to amplify them and to reinforce them with the
17 government and various arm's-length body organisations.
18 **Q.** Could you give us an idea of those types of bodies?
19 I think you mention people like NHS England, DHSC, but
20 also others?
21 **A.** So we particularly dealt with DHSC and NHS England
22 across the range of issues but we also had interactions
23 with Public Health England, as I think I reference in my
24 statement, and Number 10 and the Cabinet Office, in
25 terms of their particular interest in how services might

1 be resumed after the pandemic.
2 We interacted with regulators, so people like the
3 General Medical Council, the Nursing and Midwifery
4 Council, and also, less frequently, people like the Care
5 Quality Commission, the CQC, and similar conversations
6 were replicated in Wales and Northern Ireland by my
7 colleague.
8 **Q.** Thank you. As I'm quite sure you're aware, the Inquiry
9 has heard a fair bit of evidence about PPE quality, fit,
10 and so forth. And the focus of my questions is really
11 about, as much as we can, issues relating to procurement
12 and distribution.
13 Looking at your witness statement, can we turn to
14 paragraph 100, where you talk about personal protective
15 equipment and its distribution.
16 You say:
17 "The availability and suitability of PPE was the
18 dominant theme in the first phase of the pandemic."
19 Does that mean the dominant theme of the feedback
20 you were receiving, the insights you were receiving from
21 your members?
22 **A.** Yes, absolutely.
23 **Q.** And in what way do you say that it was the adequate --
24 availability and suitability of PPE? How did it feature
25 in that theme?

1 **A.** So there were several factors at play. I think the
2 first is every part of the health service, as well as
3 our colleagues in social care, were reporting a lack of
4 availability of PPE. That was particularly reinforced
5 that -- by certain parts of our sector, so there was
6 a perception by our members, particularly outside of
7 hospitals, that, yes, there was availability issues
8 everywhere but they were particularly challenged in GP
9 surgeries, community settings, mental health services.
10 That's the first thing.
11 I think the second thing is there was a particular
12 concern about the availability of particular equipment.
13 So concerns around masks fitted with respirators, visors
14 and goggles. There was a particular concern around the
15 availability of equipment that could fit different types
16 of faces, whether that was because of gender
17 differences, or because of ethnic minority or religious
18 observance issues. And there was also a concern, that
19 is referenced in the statement and I think we quote
20 people from the service indeed, that, actually, there
21 was an interaction between the guidance, and what felt
22 like fairly constant changes to the guidance around PPE,
23 and availability.
24 And indeed, there were some disagreements about
25 guidance. So, in particular, around aerosol-generating

1 procedures and what were and weren't aerosol-generating
2 procedures. And that both drove concern and anxiety but
3 it also drove demand for particular types of kit,
4 particularly masks with respirators, FFP3 masks,
5 irrespective of the guidance.
6 **Q.** One issue you raise in your statement is that it was
7 felt by members, I think, that the national approach to
8 PPE was felt to be focused on the acute sector.
9 **A.** Yes.
10 **Q.** And in relation to that you say this:
11 "Some members were able to secure local
12 suppliers ..."
13 You give an example of Manchester. But then you
14 say:
15 "... those local arrangements were effectively
16 nationalised ..."
17 In other words, local suppliers producing PPE to be
18 shipped across the country, which meant there was often,
19 your members were telling you, less available locally
20 than there had been before. Is there anything you can
21 say about that aspect?
22 **A.** So there's two things. I mean, I think in that initial
23 phase of the pandemic, lots of organisations, probably
24 even most organisations in the health service, had to
25 talk to other businesses that used PPE, particularly

1 face masks and gowns, and we give examples of that as
2 well in the statement. But in particular, a number of
3 organisations were able to find manufacturers who
4 switched from -- and I think the case in Manchester was
5 fixed from -- switched from manufacturing clothing to
6 manufacturing PPE. And in the, kind of, May time,
7 a communication game out from NHS England and DHSC that
8 the more significant kind of local arrangements, local
9 production arrangements in particular, actually had to
10 be made available for national supply. There was
11 a concern that, you know, Manchester may have found
12 a solution, but, you know, a different part of the
13 country was still facing real challenges and didn't have
14 the kind of manufacturing facility that colleagues in
15 Manchester were able to find.
16 And those changes, and we raised that in particular
17 with NHS England and the DHSC in May, and then we spoke
18 to them I think in June. That switch, from local to
19 national, was a real concern for people because they
20 obviously would suddenly get -- they feared they would
21 have less available for their people, their teams.
22 **Q.** You say this in your statement:
23 "New national procurement and distribution
24 arrangements in England were rapidly designed and
25 implemented", for reasons I'm sure we can understand.

1 But in terms of the feeling of your members, you say
2 that:
3 "... frustrated our members by being initially
4 unreliable, leaving ... them powerless to resolve supply
5 issues at a local level."
6 Why is it, do you think, and in terms of the
7 feedback, why is it that your members were seeking to
8 resolve things at the local level? Was there a greater
9 confidence in that or was it a necessity?
10 **A.** So, clearly, I think members understood, we understood,
11 that the colleagues who were working at pace in fairly
12 unprecedented times on this issue were doing their
13 absolute best, but/and people, both in both the initial
14 phase, gained confidence from their ability to source
15 things locally, and in the slightly later phase -- and
16 these phases were matters of weeks, as I know you all
17 know -- actually, the national system wasn't giving them
18 what they needed.
19 So there was -- the message very much was -- and
20 people understood -- the national system was scaling up
21 at real pace, but the so-called push system wasn't
22 necessarily giving people what they expected and they
23 were still having to supplement locally. So in that
24 first phase they had no choice but to contact local
25 businesses and get hold of equipment, but in that later

1 phase, when there was a much more developed national
2 system, or multiple national systems, they were still
3 having to supplement the national with local, and in the
4 example that we previously discussed in Manchester,
5 there was a fear that in those kind of examples,
6 actually that was being diverted to the national rather
7 than helping the local.
8 And these are not abstract things. These are
9 leaders, general practitioners, and general practitioner
10 leaders who weren't able, or worried they wouldn't be
11 able to provide equipment to their staff who were
12 treating patients, that they wouldn't be able to
13 properly protect their staff from contracting the virus,
14 and that's, you know, at the heart of all of this.
15 **Q.** Another issue you raise in relation to this is that NHS
16 procurement rules sometimes stood in the way of
17 opportunities and an example you give was enabling the
18 use of PPE stock held by commercial organisations in
19 their geographical area, in the hospital setting, rather
20 than waiting for the central system to procure and
21 distribute.
22 **A.** So the issues that were being raised were particularly
23 where people did find a manufacturer or some alternative
24 supply, but it didn't necessarily meet the CE
25 certification or whatever it may be that was required,

1 and that ability to rapidly deploy alternative sources
2 of equipment, again in those first couple of phases was
3 really important.
4 It was picked up at a later date, we published
5 something for members, I think it was in 2022 with NHS
6 Supply Chain, and within NHS Supply Chain's kind of
7 altered arrangements, there is something that
8 acknowledges that they had to put in place something
9 that can rapidly identify and source alternatives and
10 accredit alternatives.
11 And that wasn't, at least in our members'
12 experience, that wasn't available or easily available in
13 those early phases of five years ago.
14 **Q.** This was, you tell us at paragraph 104, something which
15 you contacted, your organisation contacted NHS
16 procurement about addressing, as you say, the challenges
17 of innovations of PPE not being able to get into the NHS
18 market due to a lack of CE certification.
19 Did you, as an organisation, support -- what was
20 your position in relation to CE certification?
21 **A.** I think we wanted to make sure, and I think we spelt
22 this out in correspondence at other times, is we wanted
23 to make sure that whatever system was required to set
24 the standards, to protect staff, protect patients, could
25 be deployed as quickly and as simply as possible, and

1 the concern that we were getting, to my understanding,
2 was that that wasn't the case, that people were being
3 told that they couldn't, you know, people felt they
4 couldn't innovate, couldn't source alternatives quickly
5 because the rules wouldn't let them do it.
6 So it was a plea for the rules to be simplified, and
7 to be made more accessible and more rapidly deployable.
8 **Q.** Is that something now, reflecting on matters, that you
9 think has happened?
10 **A.** I mean, our experience, as I've said, with Supply Chain
11 is that they have put in place a mechanism to identify
12 and support alternatives, if there's a problem with the
13 kind of preferred supply and that seems a really
14 sensible step to take and that has been tested in one or
15 two areas where there have been supply problems
16 post-Brexit, post the war in Ukraine, and that has
17 proven to be effective. Nowhere near the scale of what
18 we were facing 5 years ago, but nonetheless we've seen
19 the benefit of that.
20 **Q.** In terms of the impact of lack of supply, this is your
21 paragraph 102, you talk about your members describing
22 being unable to plan for surgical procedures due to
23 a lack of access to necessary PPE, being unable to
24 assure the safety of their staff, primary care members
25 having to rely on local shops, beauty and

1 tattoo parlours to access PPE?
2 **A.** Yes.
3 **Q.** At times having to crowd fund for their equipment?
4 **A.** Yes.
5 **Q.** And you say that these issues about inadequacy of
6 availability were raised throughout 2020. What was the
7 impact, in terms of trust and confidence or in terms of
8 anything else, on staff, was the feedback you were
9 receiving?
10 **A.** There was a profound anxiety, as I said, you know, at
11 the heart of this was the desire of leaders in
12 organisations, in trusts, leaders in primary care
13 facilities, to protect their people. So there was an
14 absolute sense of anxiety. There was a sense of a lack
15 of confidence, which only magnified that anxiety. There
16 was a lack of confidence both in the supply but also the
17 guidance.
18 There were particular areas of concern. I think it
19 became very clear in the early part of the pandemic that
20 black and minority ethnic staff were being
21 disproportionately impacted by coronavirus,
22 disproportionately represented in terms of fatalities in
23 particular, and in the April, that was a real area of
24 concern.
25 And again, that was both about total availability,

1 but also about the availability of appropriate equipment
2 or equipment that was most appropriate for the range of
3 staff that we benefit from having in the health service,
4 but yes, at its heart, it was about profound anxiety and
5 a profound understanding of the risk that frontline
6 staff were facing, and were placing themselves in, in
7 terms of caring for patients with the virus.
8 **Q.** At one point in your statement you make a particular
9 point about general practice, it's your paragraph 105.
10 **A.** Yes.
11 **Q.** You say:
12 "General practice members reported that it became
13 increasingly difficult to source clinicians willing to
14 work in hot hubs without access to PPE, with the
15 multi-source supply and confusing messaging leading to
16 low confidence in the PPE supply system."
17 What is your understanding of a hot hub?
18 **A.** So my understanding is that in the April of 2020,
19 a series of primary care, general practice-based
20 services were set up so that Covid -- patients with
21 Covid who weren't in hospital could access treatment if
22 they needed to.
23 **Q.** Your statement suggests that GPs and possibly other
24 clinicians did sometimes have to work in those hot hubs
25 without access to adequate PPE.

1 **A.** I think in the early weeks of the pandemic there were
2 absolutely reports to us, but also reports that were,
3 you know, well articulated by our trade union colleagues
4 and covered by the media where people were improvising
5 or -- in terms of PPE, or were having to re-use PPE when
6 they really rather wouldn't, but also felt that the PPE
7 they were being offered didn't necessarily meet the
8 standard, in terms of the difference between a mask with
9 a respirator and a mask without a respirator, that they
10 believed they needed for the particular examination they
11 were undertaking with patients.
12 **Q.** In April of 2020, the NHS Confederation conducted a PPE
13 survey in response to the concerns raised. Could you
14 talk us -- it's your paragraph 106 -- through some of
15 the key findings of the survey?
16 **A.** So that was a survey that was particularly aimed at
17 primary care representatives, so general practice in
18 particular. I think the two things that stood out in
19 that, to us, was that the majority, 83% of the people
20 who responded, reported that they didn't have proper
21 access to masks with filters, and that also reflected
22 the fact that demand for those masks was higher than the
23 guidance suggested, and that 73% reported a lack of
24 adequate access to goggles and visors.
25 **Q.** The survey, you say, gave the opportunity for members to

1 put in free text comment, effectively give you a bit
2 more qualitative information. This is just your
3 statement -- could we bring up, please, INQ000513763,
4 page 36, where you've exhibited some of the comments you
5 were receiving from frontline staff.
6 **A.** Yes.
7 **Q.** Paragraph 108(a), one clinician says:
8 "Had it not been for accessing PPE from schools and
9 other voluntary organisations and from our own contacts
10 with suppliers we would have run out a long time ago.
11 The national supply chain has been totally inadequate.
12 The PPE emergency line has been unreliable and failed to
13 deliver what was expected of it and we still have no
14 confidence in it. I do not believe that the initial
15 guidance (without visors or gowns) was sufficient for
16 seeing at-risk patients. These views are shared across
17 our whole PCN."
18 Pausing there, "confidence", it's a theme of your
19 statement: confidence of clinicians. Why is that so
20 important?
21 **A.** It's confidence -- as you can see it here, it's
22 confidence in terms of supply but also confidence in
23 terms of the guidance, in terms of how that supply
24 should be used.
25 Again, this speaks to the anxiety of the people who

1 were delivering care to members of our community with
2 Covid or suspected they had Covid. It was based on the
3 risk that they were very aware that they and their
4 colleagues were taking and were being asked to take, and
5 it was also, I think -- increasingly reflected the
6 particular risk factors that were becoming apparent for
7 certain members of our community, our staff community,
8 particularly our BME colleagues.
9 **Q.** Paragraph (b), please. Just very briefly. The next
10 clinician says:
11 "Having tested people previously using a level 3 PPE
12 and now testing using level 2 PPE since a guidance
13 change I feel uneasy."
14 Then the last sentence:
15 "You feel you need to as it has to be done but can't
16 help feeling concerned, more so about going home to your
17 children and feeling as though you could be putting them
18 at risk. I no longer give them a kiss."
19 Quite a vivid --
20 **A.** Yeah.
21 **Q.** -- piece of human impact. Was that sort of replicated
22 across -- was that the feeling you were getting back?
23 **A.** Yes, yes, absolutely. And of course, particularly for
24 those in leadership positions in primary care, they were
25 also practising clinicians. So they were both providing

1 a perspective in terms of the impact on their teams and
2 their colleagues, but also they had firsthand experience
3 of what the impact was on them as a practitioner.
4 And yes, this is -- you know, this both illustrates
5 the point I've been making about the changes in guidance
6 and the confidence in guidance and people's belief that,
7 actually, the guidance wasn't fit for purpose, and they
8 were having to set higher standards for themselves and
9 for their colleagues, and then -- yes, absolutely,
10 that -- (overspeaking) --
11 **LADY HALLETT:** For those who may not have been following all
12 the evidence, Mr Mortimer, your correspondent meant --
13 by level 3 PPE and level 2 PPE meant -- could you just
14 explain to the audience.
15 **A.** Of course, my Lady.
16 The difference in different types of masks. So the
17 level 3 in particular had respirators and added
18 protection and was fitted in a particular way to make
19 sure there was a proper seal on people's faces.
20 **MR STOATE:** Thank you, my Lady.
21 Finally then, sticking on the theme of guidance and
22 how it relates to procurement, can we just look at
23 paragraph (d) over the page, please. It's this:
24 "There is a strong feeling amongst clinicians [said
25 one of the respondents] that PHE guidance on PPE was

1 not, and is not, evidence based. It has altered several
2 times, seemingly mainly influenced by supply rather than
3 evidence."
4 Was that a widespread feeling?
5 **A.** I think it's hard to say how widespread it was, but yes,
6 absolutely there was a feeling that guidance and the
7 reframing of guidance, which happened a number of times,
8 was a function of available, and there was a particular
9 issue, as I think this person also refers to, around
10 specifics in guidance, particularly around what was and
11 what wasn't an aerosol-generating procedure, and some
12 difference of opinion. So PHE guidance was believed to
13 have changed, and PHE guidance was also seen to be
14 different to the guidance offered by the Resuscitation
15 Council.
16 **Q.** This clinician goes on to say:
17 "Staff just want consistency and an evidence-based
18 approach. They appreciate the truth and would have
19 been, and would still be, much more accepting of
20 a truthful approach: 'this is what you should have, this
21 is what we actually have, which is considerably less,
22 but do your best until we can upgrade you to what you
23 should have'. Our ingenuity would have helped us come
24 up with solutions."
25 This person then says:

1 "This has significantly negatively impacted the
2 frontline workforce's confidence and belief in the ...
3 centralised procurement and supply process. It will be
4 a long and difficult process ... to win that confidence
5 back."
6 Again, reflective of a wider feeling, do you think?
7 **A.** Yes, I think it was. And again, it -- there was
8 a belief that there was a lack of transparency, and
9 there was a fragility to that -- there was a fragility
10 to that confidence over the months that followed.
11 **Q.** Was there a communication issue as well, do you think?
12 **A.** Yes, I do believe there was. This colleague illustrates
13 that in terms of the kind of honesty about what the
14 supply issues were and the choices that were having to
15 be made. And I think sometimes that was reinforced
16 by -- and this wasn't just an issue with PPE, this was
17 an issue in a number of different areas -- a sense in
18 which things were announced at a nighttime briefing,
19 a ministerial national kind of lead briefing, without
20 any kind of warning to colleagues in the service or any
21 kind of preparation of colleagues in the service, and on
22 occasion that meant that people had to then the next
23 morning implement something very different for the
24 public, or -- and/or for their staff.
25 So that's confidence in that -- in the communication

1 was very fragile at times.
2 **Q.** Do you think that a lack of advance notice for
3 clinicians, as you're alluding to there, had an impact
4 upon their work, their credibility with the public?
5 **A.** Yes, at times I think it did. I mean, we saw it more
6 clearly illustrated with -- in later months with the
7 rapid changes to the vaccination programme, where the
8 public absolutely took faithfully what had been
9 announced by the government, but then the next day were
10 finding that, you know, primary care colleagues or
11 hospital colleagues, you know, were having to -- weren't
12 ready to implement the change that had been announced
13 somewhat unexpectedly the night before.
14 **Q.** Turning briefly to a couple of final issues specific to
15 procurement. In May of 2020, you wrote to Dame
16 Emily Lawson and Jonathan Marron, witnesses both to this
17 Inquiry, with the results of the survey that you told us
18 about before.
19 INQ000391177, please.
20 This is a letter from the NHS Confederation:
21 "Dear Emily and Jonathan", so Dame Emily and
22 Mr Marron.
23 Can you just tell us, what was the trigger for this
24 letter?
25 **A.** So this was a letter from my colleague Niall Dickson,

1 who was the chief executive at the time. I think there
2 were two triggers in particular, one was the
3 longstanding and kind of growing concerns that we've
4 been talking about so far this morning. The second was
5 also the moves to further centralise PPE procurement,
6 and in particular, this sense that significant -- or
7 this direction that significant local suppliers would
8 now be diverted to support the national effort rather
9 than local effort.
10 **Q.** Yes. And in fact it says in the third paragraph there:
11 "While we understand the aim to avoid NHS
12 organisations competing in a challenging international
13 market ..."
14 Because putting the alternative case, that was a,
15 we've heard evidence that that was a real concern of
16 those charged with actually procuring this stuff, one
17 I'm sure that's recognised.
18 **A.** Of course. Absolutely recognise that.
19 **Q.** You go on to say:
20 "... we strongly urge that you reassure providers
21 that attempts to achieve this will not compromise the
22 supplies they currently receive. One of our members,
23 a major acute provider, described what they see as the
24 further centralisation of PPE procurement as 'high
25 risk'. It is a view shared by others who do not have

1 confidence in the government's ability to make decisions
2 quickly, especially if sourcing from suppliers not
3 listing on existing national frameworks."
4 They may speak for themselves but we can see
5 procurement challenges very clear in that description,
6 aren't they?
7 **A.** Mm, yes.
8 **Q.** Again, where now, looking back on matters, does the NHS
9 Confederation think that the balance between allowing
10 local supply versus a centralised national supply, where
11 does that balance sit?
12 **A.** It's one of the most central questions in terms of how
13 the NHS is organised and delivered in England. It has
14 played a part in recent announcements and decisions by
15 the government in terms of how it organises the NHS in
16 England. There clearly were some decisions to allow
17 devolved nations some greater leeway to pursue, albeit
18 coordinated, but slightly more autonomous paths. And
19 I think it's always -- our reflection, and I think the
20 reflection that Niall sets out in his letter is that
21 there's just something about finding a balance in the
22 approach that's taken. And the fear about
23 centralisation, about the slow decision making, about
24 the lack of transparency which we've previously talked
25 about, which was particularly felt in the couple of

1 months prior to this letter being written, but it was
2 also part of the wider context for how the health
3 service was operating immediately prior to the pandemic
4 and arguably since then as well.
5 So that tension between local and national was
6 a constant but there were particular -- you know,
7 confidence we've talked about in our previous exchanges
8 had been eroded in the two months since the pandemic had
9 really escalated.
10 **Q.** Thank you. One particular concern in a procurement
11 context that you raise, your paragraph 115, is about
12 push stock. You touched on it briefly earlier.
13 **A.** Mm.
14 **Q.** This was in the June of 2020. You say several members
15 raised concern that the allocation of push stock of PPE
16 was not representative of what trusts required.
17 **A.** Forgive me, could I see the specific paragraph?
18 **Q.** On the screen? Yes.
19 **A.** If that would be okay. Thank you.
20 **Q.** Lawrence is probably ahead of me. INQ000513763.
21 **A.** Thank you.
22 **Q.** Paragraph 115. Thank you so much, Lawrence. Here we
23 go.
24 **A.** Yes.
25 **Q.** "In June 2020, several members raised concern that the

1 allocation of push stock was not representative of what
2 trusts required. One member said that: 'Despite sending
3 our PPE modelling and working with McKinsey, the
4 quantities that came through, the push model is still
5 not adequate in terms of quantity. This leads to Trusts
6 being forced to source their own materials'."
7 Again, possibly self-explanatory but the real world
8 NHS frontline consequence of what looks like a sort of
9 technical procurement issue?
10 **A.** Yes, I mean, a huge amount of work had been done with
11 McKinsey. Lots of -- I think a number of our members
12 had supported McKinsey in trying to design this push
13 system for trying to model demand and issue stock out to
14 organisations, but as our colleague references there,
15 that didn't meet what was actually happening in
16 practice, and there were still occasions where people
17 had to use the kind of local routes that we've described
18 to supplement what was coming thorough that national
19 system.
20 **Q.** Quite a vivid description from the next member in your
21 paragraph here. I don't know whether this is hyperbole
22 or true but the point is made:
23 "We seem to get delivered what is available rather
24 than what we need. For example I have several years'
25 supply of visors now, but short of gowns. No ability to

1 return them (so swapping in [local resilience forum]),
2 but delivery is not related to need."
3 Do you think that was an issue, clearly it's acute
4 in the June of 2020 -- was that an issue that improved?
5 Was that the feedback you got?
6 **A.** So any hyperbole I think would reflect the kind of
7 concern --
8 **Q.** Absolutely.
9 **A.** -- that we've discussed, and that kind of heightened
10 concern in particular about the risk that staff were
11 bearing. Yes, absolutely, as we headed into the summer
12 and particularly into the autumn, confidence increased,
13 and the kinds of resilience of supply, the adequacy of
14 supply improved. There were bumps along the road in
15 that, of course, and some fairly significant ones, but
16 yes, the confidence did increase. But even in June, as
17 this colleague is illustrating, there was -- there were
18 imperfections in the system, particularly in terms of
19 how the push stock operate or the push system worked,
20 rather. And this requirement for people still to source
21 stock, or to swap stock through the local resilience
22 forums, so the kind of coordinating mechanism there was
23 in the various places around the country.
24 **Q.** Just one other document to show, you, please. This is
25 August of 2020, your then chief executive sent another

1 letter to Dame Emily Lawson --
2 **A.** Yes.
3 **Q.** -- seeking reassurance on PPE supply and stock.
4 Lawrence, as ever, is ahead of me, but for the
5 transcript it's INQ000391178.
6 You can see here another letter:
7 "Dear Emily ..."
8 That's Dame Emily Lawson, isn't it?
9 **A.** Yes.
10 **Q.** 12 August. Here recognising -- second paragraph:
11 "... significant progress has been made in recent
12 months to move away from 'hand-to-mouth' PPE supply
13 chain but are concerned [about a] latest change ..."
14 And a concern there about needing to accompany that
15 with "mitigating measures".
16 Again, a similar sort of -- seems to be part of the
17 same debate around local versus national.
18 **A.** So, as you've highlighted, Niall had a -- Mr Dickson had
19 a couple of exchanges with Emily, and in the June, Emily
20 and Lord Deighton met a cross section of members to talk
21 about the issues and to kind of have firsthand feedback
22 about the things that we've talked about largely, which
23 are reflected elsewhere in the statement.
24 I think what we saw in the August was changes to the
25 delivery system, but a concern among members: that the

1 changes -- that they hadn't had sufficient assurance
2 that the changes that were being made could cope with
3 surges in Covid, particularly if they were on a regional
4 rather than a national basis; the plan, and there were,
5 you know, plans developing in that summer to step back
6 up elective activity; and then the impact that we knew
7 came every winter; giving flu vaccinations but giving
8 flu vaccinations in the context of the pandemic; and
9 urgent and emergency care demand, but obviously managing
10 that kind of demand for urgent and emergency care, the
11 stepping up of elective care in the context of
12 a pandemic.
13 So again, it spoke to this point about confidence.
14 If you needed to, could you revert the system back to
15 24/7 deliveries? If you needed to, have you got
16 sufficient stock to be able to respond to these kind of
17 increases in demand that we expect or we fear may come
18 through the autumn and into the winter?
19 **Q.** Understood. Just looking briefly at the next paragraph:
20 "Furthermore [says Mr Dickson], we would welcome
21 clarification on the impact, if any, the loss of
22 fifty million face masks via the £252m Ayanda Capital
23 contract will have on the NHS."
24 Just pausing there, you may be aware that this
25 module of the Inquiry is looking at a range of different

1 contracts. I'm not asking you to comment in my
2 questions here on the correctness or otherwise of those
3 particular figures, but it goes on -- what the letter
4 goes on to say is:
5 "The incident highlights why ensuring the PPE
6 distribution network needs to remain robust and
7 distribution immediately escalated if required.
8 Confirmation that any impact from this incident on
9 supply chains has been resolved and steps that have been
10 taken to prevent future incidents of this scale would be
11 most welcome."
12 What I'm reading here is an example of a particular
13 procurement issue arising, possibly in the media and
14 being well publicised, and your organisation, through
15 its chief executive, feeling the need to raise this at
16 the highest level of NHS England; is that right?
17 **A.** Yes, it is. And again, I -- looking at it now, it
18 speaks to that question of confidence. And whilst the
19 situation was improving, situations like this -- and
20 you're absolutely right, I don't know the detail, we
21 wouldn't know the detail of the specific contracts or
22 the organisation, even, that's named, but that being
23 reported in the media impacts on the confidence of
24 people in hospitals and mental health and community
25 facilities, in primary care, and there's something about

1 reassuring them that the system can withstand and can
2 cope with the loss of what feels like a significant
3 element of product that people will be relying on,
4 particularly in that context where there may have been
5 some thankful receding of the pandemic over the summer
6 of 2020, but people were anticipating further surges as
7 well as all the other things I talked about in terms of
8 stepping back up some level of normal services.
9 So, yeah, it was absolutely about reassurance. It
10 was absolutely about confidence and about wanting to
11 understand that the system could cope with the loss of
12 50 million face masks, for example.
13 **Q.** I want to ask you one very brief question about a topic
14 about which we heard quite a lot of evidence yesterday:
15 oxygen supply.
16 **A.** Okay.
17 **Q.** It's your paragraph 138, and just puts a slightly
18 different piece of the picture in. You say:
19 "Mental health wards in many places had experienced
20 difficulty in accessing enough oxygen tanks to support
21 Covid-19 patients."
22 Why was that a particular problem, and was it one
23 that you were receiving feedback was widespread?
24 **A.** So my understanding is that that's because patients who
25 would normally -- who needed non-mental health care,

1 other physical health needs, who would normally transfer
2 into an acute hospital because of the pandemic, the
3 threshold for that transfer had changed, and they were
4 being cared for in their mental health facility, and the
5 use of oxygen just isn't as widespread in mental health
6 clinical settings as it would be in an acute hospital.
7 I'm not able to comment on how widespread an issue
8 that was. If that was available, we could inform the
9 Inquiry afterwards, but I can't comment on that at this
10 stage.
11 **Q.** Understood.
12 Final question: any further reflections upon the
13 evidence that you've given today? Anything else you'd
14 like to add?
15 **A.** I think clearly the recommendations from my Lady will be
16 really important in terms of the planning for future
17 such events, but clearly organisations need to be taking
18 action now, and we have seen some evidence of that, and
19 I think that is important to emphasise. Clearly other
20 reviews have taken place. I've already referenced some
21 of the things that Supply Chain are doing. And that
22 includes I think, importantly, not just the
23 practicalities, the logistics and ordering systems and
24 procurement and so on, but it's also about engagement,
25 it's also about that -- building that relationship with

1 organisations and particularly with clinical teams.
2 I think that's the first piece.
3 I think the second bit then is the need, as we've
4 touched on, to understand how the people who were
5 delivering services felt, the risk that they were
6 carrying, the anxiety that they felt for themselves, for
7 their colleagues and, as was illustrated, for their
8 families. And that was true for all parts of the
9 workforce but it was particularly true for people of
10 colour, for ethnic minority colleagues, for colleagues
11 who joined the NHS in such numbers from overseas as
12 well.
13 And that disconnection, in terms of guidance,
14 sometimes -- and steps were taken to resolve it, and I'm
15 not minimising the things that particularly colleagues
16 at NHS England did to try to respond to that during the
17 course of the spring, but that's a really important
18 lesson, that impact on people on the front line and that
19 need for transparency but also responsiveness to that
20 impact on colleagues.
21 **MR STOATE:** Thank you very much.
22 My Lady, those are my questions.
23 **LADY HALLETT:** I don't think there are any Rule 10
24 questions.
25 **MR STOATE:** No.

1 **LADY HALLETT:** Thank you very much indeed for your help,
2 Mr Mortimer, I'm very grateful.
3 **THE WITNESS:** Thank you.
4 **LADY HALLETT:** And what you said there about understanding
5 people on the front line, that seemed to echo -- I don't
6 know if you saw the evidence of Dr Kevin Fong about what
7 it was like to be on the front line and how important it
8 was to feed back to people at the centre exactly what
9 was going on.
10 **THE WITNESS:** Yes.
11 **LADY HALLETT:** So thank you very much indeed for your help.
12 **THE WITNESS:** Thank you, my Lady.
13 **MR STOATE:** My Lady, the next witness, if I may, is
14 Rosemary Gallagher.
15 **LADY HALLETT:** Welcome back.
16 **THE WITNESS:** Thank you, my Lady.
17 **MS ROSEMARY GALLAGHER (sworn)**
18 **Questions by COUNSEL TO THE INQUIRY**
19 **MR STOATE:** Thank you.
20 Good morning, Ms Gallagher. Could you give the
21 court your full name, please.
22 **A.** Rosemary Gallagher.
23 **Q.** Gallagher, forgive me, thank you.
24 This is not your first time here and not your first
25 witness statement but for the benefit of this module you

1 have very helpfully provided a statement with the
2 Inquiry reference INQ000553817. It is in fact a joint
3 statement produced with your colleague Suman Shrestha?
4 **A.** That's correct.
5 **Q.** It's 37 pages long, and you signed it on 13 January of
6 2025?
7 **A.** That's correct.
8 **Q.** And is that statement true to the best of your knowledge
9 and belief?
10 **A.** It is.
11 **Q.** Thank you very much. By way of very brief
12 reintroduction for this module, you are the professional
13 lead for Infection Prevention and Control, and the
14 nursing sustainability lead at the Royal College of
15 Nursing; is that correct?
16 **A.** That's correct.
17 **Q.** You were appointed to that role in July 2009?
18 **A.** Yes.
19 **Q.** You say you currently sit on a number of external
20 national committees aligned with the Royal College of
21 Nursing, including the Royal College of Physicians'
22 Patient Safety Committee, and the NHS England Emergency
23 Planning Resilience and Response Clinical Reference
24 Group, amongst others?
25 **A.** That's correct.

1 **Q.** Your role is UK wide, and the RCN and yourself respond
2 to the needs of each country of the United Kingdom as
3 they arise?
4 **A.** Yes, as a UK-wide organisation.
5 **Q.** Thank you very much.
6 In terms of the role of the Royal College of Nursing
7 during the pandemic, its members work in a variety of
8 hospital and community settings in the NHS and
9 independent sectors; is that correct?
10 **A.** That is correct, yes.
11 **Q.** And there are over 300,000 members employed within
12 the NHS?
13 **A.** Yes.
14 **Q.** The focus of these questions will be, firstly, the
15 impact of any concerns that the RCN received arising in
16 particular from the procurement of PPE; and, secondly,
17 looking at any lessons which might be learned from yours
18 and the RCN's perspective. Okay?
19 Looking first at the section of your statement
20 entitled "Reports of shortages of PPE", paragraph 11,
21 the first section you talk about here is the RCN support
22 service, RCN Direct.
23 **A.** Yes.
24 **Q.** Which consists, you say, of a call centre and an online
25 platform. Can you give us an overview of what was

1 coming in to that call centre? What kind of concerns,
2 what kind of queries were coming into that call centre?
3 This is your paragraph 11.
4 **A.** Yes, so at that time the predominant theme of questions
5 and queries and concerns coming from members related to
6 the pandemic response, but specifically, personal
7 protective equipment, either access to, or the type of
8 PPE that they were being advised to used.
9 **Q.** And the time we're talking about here is March of 2020,
10 is it?
11 **A.** Yes, that's when the concerns really started to
12 escalate.
13 **Q.** The call centre and online platform received more than
14 3,500 queries just relating to PPE, did they?
15 **A.** Yes, that was an unprecedented number of enquiries that
16 we received.
17 **Q.** 1,300 of those reported lack of access and 700 related
18 to general shortage or a lack of specific items; is that
19 right?
20 **A.** Yes, that's correct.
21 **Q.** You say that your members were telling you things like
22 being able to -- being unable to access a specific type
23 of FFP3 respirator that they'd been fit tested for?
24 **A.** That's correct, yes.
25 **Q.** Is that quite a common concern?

1 **A.** It was at that time. Under business as usual, not every
2 member of the healthcare workforce would have been
3 fit tested for respiratory protective equipment or an
4 FFP3 mask. So there was huge demand for the workforce
5 to be upskilled in this. Staff that had been used to
6 wearing FFP3 masks suddenly found themselves without the
7 normal type of respirator they had been fit tested for
8 so that necessitated a change and additional fit
9 testing. And actually caused them significant concern
10 because they were used to one particular mask that we
11 were able to access, but then all of a sudden, that
12 market disappeared.
13 **Q.** You say there was also a problem in finding alternative
14 respiratory protective equipment or RPE where members
15 couldn't tolerate wearing those specific brands; is that
16 right?
17 **A.** That's correct. So we had always, and continue to work
18 on the principle that if one mask doesn't fit correctly
19 or for some reason is uncomfortable, that we would seek
20 an alternative. So that was extremely difficult in
21 those early stages.
22 **Q.** You say there were also concerns about out-of-date PPE,
23 some of which broke upon use or had physically degraded?
24 **A.** Yes, we had many concerns from members around material
25 degrading but particularly the nose bands, and actually,

1 members reported some really quite distressing incidents
2 of respiratory irritation there they were inhaling the
3 fibres from these degraded masks. So there was real
4 concern, even though the process of re-labelling
5 out-of-date PPE was explained at the senior level, but
6 actually what they were receiving was not fit for
7 purpose and potentially causing them harm.
8 **Q.** You say in your paragraph 12:
9 "A small number of members reported being asked to
10 shave facial hair to be fit tested, when this was
11 inappropriate for religions reasons."
12 **A.** That's correct, so we did receive some reports of this.
13 Under business-as-usual circumstances, those staff with
14 beards that were -- that wanted to retain their beards
15 would have been offered alternative respiratory
16 protective equipment through different types of hood
17 respirators, for example, but we simply didn't have the
18 access to those in the numbers that we needed.
19 **Q.** In response to some of these concerns, the Royal College
20 of Nursing undertook what you describe as two extensive
21 surveys of its members working across all health and
22 social care sectors, first in April and then in May of
23 2020?
24 **A.** That's correct, yes.
25 **Q.** And this was specifically about the use of and

1 availability of PPE, to gain a better understanding of
2 the depth and scale of the problems that were being
3 reported through --
4 **A.** Mm, yes.
5 **Q.** -- these call centres and online platform. Could you
6 give us a flavour of the finding of the first
7 statement -- the first survey, forgive me -- published
8 in April 2020?
9 **A.** So a significant number of members at that time reported
10 being asked to re-use what had been designated as
11 single-use PPE. So single-use means single-use -- staff
12 have always had to work to that. So at this time to be
13 asked to re-use single-use PPE, which was there for
14 their protection, was a significant cause of concern to
15 them.
16 A lot of staff reported a lack of eye protection.
17 So we were very concerned around potential infection
18 through the conjunctiva at the time, and therefore,
19 staff understood that this was required as part of the
20 PPE ensemble, but wasn't able to be provided.
21 **Q.** There was a second survey -- that was the first survey
22 of April?
23 **A.** Yes.
24 **Q.** And it was extensive, wasn't it? It was more than
25 13,500 members --

1 **A.** Yes, it was a very good response, yes.
2 **Q.** The second survey, in May, still a significant number of
3 responses, more than 5,000. What were the key findings
4 of that survey?
5 **A.** So the key findings of that survey were that some
6 improvements had started to be noted in terms of the
7 supply of PPE. There were still reports of staff being
8 asked to re-use what had been designated as single-use
9 PPE and also, as with the first survey, staff reported
10 concerns or a lack of confidence when raising issues
11 with their managers that, actually, these would be
12 resolved.
13 So that was a persistent theme through both surveys.
14 **Q.** You say that 34% of respondents felt pressure to care
15 for individuals with possible or confirmed Covid-19
16 without adequate protection, and for some 56% of
17 respondents from ethnic minorities felt pressure to work
18 without the correct PPE; is that right?
19 **A.** That's correct.
20 **Q.** You, I think, shared the results of these findings, if
21 I may say, far and wide. You give the list in your
22 statement: DHSC, the Prime Minister, Scottish Cabinet
23 Secretary for Health and Sport, members of the Welsh
24 Senedd, Audit Wales, the HSE, and NHS England; is that
25 right?

1 **A.** That's correct.
2 **Q.** You say that on 17 April in response to the first survey
3 findings, HSE acknowledged the survey results and noted
4 that it was working closely with other government
5 departments, including the DHSC, to facilitate efficient
6 procurement and distribution and effective PPE.
7 But you say, and this is your paragraph 19, in their
8 response, NHS England noted that there had never been
9 a shortage of respirator masks or gowns and requested
10 more data.
11 **A.** That's correct.
12 **Q.** What were your members saying to you about this?
13 **A.** Well, the survey findings reported our members' lived
14 experience of the provision and adequacy of PPE at that
15 time. The survey questions didn't tell us why they
16 responded in the way that they did, but that that was
17 their experience at that time.
18 **Q.** At this point, did you have any concerns around the IPC
19 guidance or modelling that related to some of these
20 findings or in particular, the question of whether there
21 was in fact a shortage of respirator masks or gowns?
22 **A.** Yes, many members had expressed concerns that the IPC
23 guidance was actually driving the supply of PPE. So
24 rather than the use of PPE being driven by what nurses
25 on the front line felt that they needed, based on their

1 risk assessment, that, actually, the way the IPC
2 guidance was structured at that time was leading them to
3 wear, for example, surgical face masks as opposed to an
4 FFP3 mask outside of an aerosol-generating procedure, so
5 a high-risk procedure. The modelling for the supply and
6 demand of PPE, I understand was based on the IPC
7 guidance, according to some statements in Module 3, and
8 therefore, that had a significant impact because the IPC
9 guidance didn't take into account, for example, if
10 you're wearing an FFP3 mask the Health and Safety
11 Executive advise a one-hour continuous wear time, after
12 which you're expected to take your mask off to support
13 your skin health, for example, and then to apply a fresh
14 mask.
15 Staff also were so concerned around what they
16 perceived as a lack of personal protective equipment,
17 predominantly masks, and they knew they would have to
18 remove all their PPE to go to toilet or to have -- to
19 take breaks, that they actually put off going to the
20 toilet because they were so concerned that they would
21 deplete the stocks or that their colleagues coming into
22 the next shift might not actually have enough personal
23 protective equipment.
24 So the whole considerations around how much PPE you
25 use, not just on paper for a shift, but actually the

1 real-life on and off constantly during the day, they
2 felt just wasn't taken into account.
3 **Q.** Those sound like the sorts of experiences best
4 understood at the front line?
5 **A.** Yes.
6 **Q.** When to use PPE, how long for, how it affects a toilet
7 break or a --
8 **A.** Yes.
9 **Q.** -- break to have a glass of water or something to eat.
10 I want to turn to how that, in your view, and the
11 RCN's view more generally, fed into the procurement
12 process and how it might do so in the future.
13 **A.** Right.
14 **Q.** In your witness statement at paragraph 23, as well as
15 reports, as we've discussed specifically on the
16 availability of PPE, your members were raising a number
17 of issues about procurement, including -- you may have
18 touched upon this already -- equipment not being fit for
19 purpose, and you provide a quote here, which I may just
20 read briefly, from a member:
21 "So far we have had 4 different types [of mask] from
22 2 different manufacturers, none have been clinically
23 acceptable, they simply do not fit. I am aware that
24 contracts have already been signed to purchase. When
25 I have fed back to the Cabinet Office that they don't

1 fit, I was told any mask was better than nothing.
2 Another company on a call wanted to know how the NHS fit
3 test masks, and when I questioned them on which Trusts
4 were already using their FFP3 masks, or had evaluated,
5 they said none, yet they had also been contracted to
6 supply into the NHS."
7 So a flavour of that kind of concern.
8 **A.** Yes.
9 **Q.** Another one you've already told us about, I think, or
10 perhaps this is different, poor packaging of PPE which
11 in turn impacted access to adequate PPE for nurses and
12 other healthcare workers. Yes?
13 **A.** That's correct.
14 **Q.** And then this, what you describe as a lack of clinical
15 engagement in procurement decisions. This at your
16 witness statement paragraph 23(b). Again, to give
17 a flavour, a member said this to you or your
18 organisation:
19 "The clinical engagement is being done AFTER the
20 purchasing decisions have already been made. It is
21 paying lip service to it and it is too late once
22 decisions have already been made based on tech specs,
23 NOT clinical specification and evaluations."
24 **A.** That's correct.
25 **Q.** So that's the concern and that's what was being

1 expressed to, and through, your organisation?
2 **A.** That's correct.
3 If I might just come in on that, the specification
4 for a product, whether it's a mask, a ventilator,
5 a wound dressing, is really just a description of the
6 product and, where applicable, what standards it has to
7 meet. It's not the same as whether it can actually be
8 used well in practice.
9 So the procurement nurses that have specialist
10 skills as clinicians, understand how these products are
11 used in practice.
12 So whilst, on paper, it might say that it does it,
13 what they were finding was that when it actually reached
14 them on the front line, the products just simply weren't
15 fit for purpose.
16 **Q.** Well, I want to ask you, then, about procurement nurses.
17 It's probably well understood that nurses deliver
18 a significant percentage of physical and psychological
19 care to patients.
20 **A.** That's correct.
21 **Q.** But within the nursing profession, what is a specialist
22 procurement nurse?
23 **A.** So a specialist procurement nurse is -- it's
24 a relatively new type of specialism. These are
25 registered nurses that have worked in clinical

1 environments, often from operating theatres to intensive
2 care units, renal care, other areas, that have moved
3 into support procurement teams in NHS trusts to make
4 decisions about the right products for use in clinical
5 practice.
6 As nurses, we use the most consumables in the NHS,
7 and we use them in many different ways. So these nurses
8 are not only experts in how products are used, but they
9 also are aware of how clinical practice is changing and
10 adapting over time, and often the products don't -- or
11 there is a risk that products might not keep up with
12 clinical practice.
13 Also, clinical procurement teams may not actually
14 have that clinical experience, so the word "clinical"
15 can be a bit of a misnomer within the procurement world.
16 So these are experienced clinical nurses that understand
17 how products are used and can advise on benefits or
18 contradictions to how they are used in clinical
19 practice.
20 **Q.** Something we heard a bit about yesterday in the evidence
21 to the Inquiry about medical technology was something
22 described as a human factor approach.
23 **A.** Yes.
24 **Q.** Human factors approach. Is that something that --
25 **A.** So specialist procurement nurses would understand in

1 depth the human factors aspects of how products are used
2 and what makes it a good product or not. They would
3 also be very knowledgeable on, for example, the
4 implications for buying a poor-quality product. Because
5 we may use more or we may not use them at all.
6 So wastage is a real issue within the NHS, and
7 something that procurement nurses are very mindful of,
8 and during the pandemic, to have these items of PPE
9 delivered to their trust that were clearly not fit for
10 purpose, to see the huge amount of waste that occurred
11 as a result of that, was actually really distressing to
12 nurses, because they knew that if they had been involved
13 at an earlier stage in assessing products that were
14 coming to the UK, that they may have been able to avoid
15 a lot of that waste, actually, you know, one, coming
16 into the country, or, two, actually reaching the front
17 line.
18 **Q.** Had specialist procurement nurses been working within
19 the NHS procurement framework or system prior to the
20 pandemic?
21 **A.** They had. So from 2012 the royal college had been
22 involved in work with the NHS National Customer Board,
23 as it was at that time, and working through a clinical
24 reference group had been part of a piece of work,
25 a proof of concept around testing the benefits of

1 clinical evaluation led by nurses.
2 So this was aimed at ensuring that the products that
3 nurses had to use were fit for purpose, that they were
4 of the correct quality and didn't harm either patients
5 or staff, potentially.
6 The work was undertaken through the clinical
7 evaluation team, and was so successful that it was
8 adopted as a core criteria of how the NHS Supply Chain
9 worked with the delivery of consumables across the board
10 to the NHS.
11 **Q.** In your statement you suggest that this was something
12 you were raising -- or I should say the opposite, the
13 lack of involvement during the pandemic was something
14 you were raising as a concern?
15 **A.** Yes.
16 **Q.** You give, at paragraph 24, a particular example of
17 a conference call with the Cabinet Office and
18 NHS England, which you say took place as a result of
19 member concerns following the publication of an interim
20 PPE procurement structure. I just want to look briefly
21 at that structure, please.
22 INQ000417641.
23 This is a document you provided with your statement.
24 **A.** Yes.
25 **Q.** If I may say, a slightly bewildering -- I think we call

1 it an organogram, headed "PPE -- Interim Team
2 Organisation (As of 1 July 2020".
3 My suggestion -- but, please, your evidence, please
4 say -- it looks like an awful lot of procurement
5 professionals and no clinicians, in particular no
6 nursing presence.
7 **A.** That was my take from this, as it was how the members
8 interpreted it as well.
9 **Q.** So you had concerns about that as soon as you saw this?
10 **A.** I did. I became aware of this towards the end of June
11 and immediately raised it on behalf of specialist
12 procurement nurses, because we knew the value that
13 nurses could bring to the selection of products, given
14 what a serious situation it was in. And it was actually
15 really upsetting for those nurses and for us to find
16 that the clinical voice was completely absent from this
17 organisational organogram.
18 **Q.** Thank you.
19 Thank you, Lawrence, we can get that down.
20 By way of follow-up to that conference call, you say
21 later on you emailed NHS Improvement, on 16 July,
22 forwarding a large number of member concerns.
23 **A.** Yes.
24 **Q.** You say you reminded NHS England that there were
25 a number of experienced RCN members who would be happy

1 to support evaluation of PPE procurement either via the
2 DMC, which I think is the decision management --
3 A. Decision making.
4 Q. Decision making committee, forgive me, or as part of
5 various groups looking into different aspects of current
6 and future procurement of PPE.
7 "This [you say] was based on previous RCN
8 involvement in procurement and clinical evaluation
9 methodologies ..."
10 Which we've just been discussing.
11 A. Yes.
12 Q. "... of consumables used by nurses to deliver patient
13 care. To my knowledge, I do not believe that this offer
14 was ever taken up and I am unable to locate any
15 response."
16 Looking forward, one of the key lessons, I think,
17 that you say should be learned is about the centrality
18 of the involvement of the frontline perspective in the
19 procurement process; is that right?
20 A. Absolutely. We had learnt from the lessons in Ebola
21 that those people that use PPE need to receive education
22 on its use, so donning and doffing. It's absolutely
23 critical that they're involved in decisions about what
24 is bought, what is clinically acceptable in practice.
25 So that, in addition to our experience through the

1 clinical evaluation team around the benefits of clinical
2 evaluation means that in a future incident it's
3 absolutely imperative that frontline healthcare workers
4 are involved, and that we can also look at the needs of
5 our colleagues that work with us. So they're very
6 conscious of different face shapes, different sizes,
7 religious and cultural needs, for example, and can bring
8 that into the discussion.
9 Q. Just one other brief issue in relation to the work of
10 specialist procurement nurses. This exchange you were
11 having with NHS England in the July of 2020, you say
12 that you took the opportunity to, in your
13 correspondence, to explain that you'd been contacted by
14 one trust who'd been -- who had raised concerns over the
15 complexity of reporting issues with PPE --
16 A. Yes.
17 Q. -- as some items needed to be reported via the MHRA and
18 others via different routes --
19 A. Mm.
20 Q. -- and how time consuming you say this was if there were
21 multiple issues. And then this phrase you use:
22 "... adding unbearable pressures to specialist
23 procurement nurses supporting NHS Trusts ..."
24 A. Yes.
25 Q. "... and wider systems at this time."

1 So not looking at procurement nurses from the point
2 of view of before it's procured, but now you're there on
3 the front line, there are issues to report. What does
4 this tell us?
5 A. So nurses are very aware of the NMC code which requires
6 them to report and address issues relating to safety or
7 quality of care, and that also extends -- would extend
8 to reporting issues relating to PPE, in practice.
9 Procurement nurses were under extreme pressure
10 during the pandemic. They felt significant
11 responsibility for ensuring that the correct and
12 acceptable PPE was distributed to their colleagues in
13 practice, so this was very, very personal to procurement
14 nurses.
15 They took the time, on top of everything else they
16 were doing, to report issues, but they had to be
17 reported through multiple routes, as you said. And it
18 took an extraordinary amount of time to do that. And on
19 top of that, they rarely heard anything back.
20 So in terms of learning for the Inquiry, what we and
21 specialist procurement nurses would like to see in the
22 future is a one-stop reporting system where the
23 information is taken out, so you only have to report
24 that particular issue through one particular portal.
25 And this reporting and the pressure, of course, was

1 felt across every NHS trust in England and it would have
2 happened in the four countries as well. So it
3 represented a huge amount of time, but it was felt to be
4 completely necessary.
5 Q. Thank you.
6 In terms of the -- just finally, then, you've
7 provided a very reflective set of lessons learned and
8 recommendations in your witness statement. I've only
9 focused on a few of those today. Obviously your full
10 witness statement will be published --
11 A. Yes.
12 Q. -- and considered very carefully. Is there anything
13 else reflecting on the issues that we've touched upon in
14 our discussion here that you want to say, as a lesson
15 learned or a key reflection from what the experience of
16 you and your members was during the pandemic?
17 A. Yes, thank you. I'd like to reinforce the need for the
18 clinical voice and specialist nurses to be part of all
19 decision making relating to the procurement of PPE, but
20 also that that process is transparent and formalised to
21 avoid variation at a time of escalation.
22 As with Module 3, we believe that we urgently need
23 innovation and respiratory protective equipment, to
24 avoid the issues that we've seen time and time again
25 through the pandemic around not just the supply of PPE,

1 but the need to fit test on multiple occasions. But
2 that PPE has to be developed so that it's not just
3 acceptable for staff but also acceptable for patients.
4 And the other point, based on our findings from our
5 survey, is that the needs and experience of our black
6 and minority ethnic staff in all care settings are
7 explored, captured, and addressed so that we can learn
8 from their experience to ensure that they are fully
9 inclusive in the future, as one team, but specifically
10 in relation to the use of personal protective equipment.
11 **MR STOATE:** Thank you very much indeed.
12 My Lady, those are my questions.
13 **LADY HALLETT:** Thank you, Mr Stoate.
14 Mr Wilcock. Mr Wilcock is over there.
15 **Questions from MR WILCOCK KC**
16 **MR WILCOCK:** Good morning.
17 **A.** Good morning.
18 **Q.** Ms Gallagher, I'm asking you questions on behalf of the
19 Northern Ireland Covid Bereaved Families for Justice,
20 and what I'd like to do is, first of all, just show you
21 an extract from the Northern Ireland Audit Office report
22 which is at INQ000348882, and if we could go to page 39,
23 paragraph 7, please.
24 This is an extract from, as I say, the Northern
25 Ireland Audit Office report entitled "The Covid-19

1 pandemic supply and procurement of PPE to local
2 healthcare providers."
3 And you can see at paragraph 7 it states that the
4 RCN have raised issues over the guidance, by which
5 earlier paragraphs make clear it means the core UK
6 infection and control guidance, in early 2020 with the
7 Department of Health and the Public Health Agency. It
8 also wrote to HSE -- Health and Safety Executive
9 Northern Ireland in late 2020, stating that:
10 "'We do not believe our members are being adequately
11 protected with the current UK guidance that is being
12 adopted within Northern Ireland', as well as
13 highlighting members' views that employers were
14 interpreting guidance based around PPE availability,
15 instead of what was required to protect staff".
16 Now, you've told us about the concerns that your
17 members had across Great Britain, so my questions are
18 obviously going to ask you to widen that to Northern
19 Ireland, as well. And the first question is, is it
20 correct that the RCN membership in Northern Ireland were
21 raising the concerns that their employers were
22 interpreting guidance based around PPE availability at
23 this time?
24 **A.** We did receive concerns from members across all four UK
25 countries, including Northern Ireland, and certainly our

1 country directors were very tuned in to their regional
2 nurses and what was coming from the front line with the
3 respective countries. So in answer to your question,
4 yes, we did have concerns raised by members in Northern
5 Ireland.
6 **Q.** And do you have anything to add to what you told
7 Mr Stoate about what you meant by the phrase "based
8 around PPE availability rather than need"?
9 **A.** Our members at the time were very concerned around the
10 availability of PPE, and this coincided with a change in
11 the IPC guidance and the downgrading of SARS-CoV-2 from
12 a high-consequence infectious disease, and there was
13 real concern across the UK from nurses that this was
14 predicated on availability of PPE, rather than based on
15 risk. So that was a common perception at that time.
16 **Q.** Yes, thank you. And then finally, in terms of how the
17 concerns were collected and recorded by the RCN within
18 Northern Ireland, was that also through the RCN Direct
19 service and the two surveys in April and May that you've
20 told us about or was there something else as well?
21 **A.** So definitely with the surveys, members from Northern
22 Ireland contributed. RCN Direct is a UK-wide portal, so
23 members would have accessed through that one-stop shop.
24 **Q.** Yes.
25 **A.** It's possible that members also contacted their regional

1 leads within Northern Ireland separately, but I don't
2 have access to that information at this time.
3 **Q.** In any event, the concerns were there?
4 **A.** Yes, absolutely. Yeah.
5 **MR WILCOCK:** My Lady, thank you very much.
6 **LADY HALLETT:** Thank you, Mr Wilcock.
7 Ms Parsons, right over there.
8 **Questions from MS PARSONS**
9 **MS PARSONS:** Thank you, my Lady.
10 Good morning, Ms Gallagher, I represent the Bereaved
11 Families for Justice Cymru.
12 Just one topic, PPE and IPC guidance, so it is
13 something that you have touched on already, both with
14 Counsel to the Inquiry and just a moment ago, and
15 counsel for the UK Bereaved Families.
16 You've expressed concern, Ms Gallagher, in your
17 statements to this Inquiry that IPC guidance was not
18 based on aerosol transmission of the virus, even though
19 there was evidence that such a virus was transmitted by
20 aerosol.
21 **A.** Yes.
22 **Q.** In your opinion, as a professional lead for IPC, why was
23 aerosol transmission overlooked when it came to
24 developing and updating IPC guidance?
25 **A.** The IPC guidance at the beginning of the pandemic was

1 led by the four nations' national IPC teams and members
2 of Public Health England. I'm not able to speak as to
3 why they didn't draw or adapt the guidance as the
4 evidence was emerging. All I can say is that, from
5 a royal college perspective, we viewed the risk of
6 aerosol transmission as something that was very real,
7 and advocated and lobbied for our members based on that
8 principle.
9 **Q.** And to put it another way, or to ask you this to follow
10 on, Ms Gallagher, to what extent was the guidance due to
11 a shortage of appropriate PPE such as FFP3 masks?
12 **A.** We know that there was a shortage, but I'm not able to
13 say exactly how short we were. We weren't privy to the
14 information around the exact level of supplies and
15 stocks of PPE at that time. What we heard from -- was
16 from our members telling us that there were insufficient
17 supplies, or that the supplies were really, really tight
18 in terms of a just-in-time delivery, with organisations
19 and care homes sharing if they had surplus between them.
20 So it was a very mixed and confusing picture at that
21 time.
22 **MS PARSONS:** Thank you very much, Ms Gallagher.
23 Thank you, my Lady.
24 **LADY HALLETT:** Thank you, Ms Parsons.
25 Thank you very much indeed, Ms Gallagher. I suspect

1 I might be calling upon you again.
2 **THE WITNESS:** I look forward to it.
3 **LADY HALLETT:** I can't say that you're off the hook already
4 but thank you very much for your help.
5 **THE WITNESS:** Thank you very much, my Lady.
6 **LADY HALLETT:** Very well, time for a break?
7 **MR STOATE:** Just before we do, made, if I may, for the
8 purposes of the transcript and those following online,
9 I'm informed that the correct page reference for the
10 document referred to by Mr Wilcock, INQ00034882, is in
11 fact page 69, not page 39.
12 **LADY HALLETT:** Thank you very much. I shall return
13 at 11.35.
14 **(11.20 am)**
15 **(A short break)**
16 **(11.35 am)**
17 **LADY HALLETT:** Mr Sharma.
18 **MR SHARMA:** My Lady, the next witness is Lord Deighton.
19 **LORD DEIGHTON (affirmed)**
20 **LADY HALLETT:** I do hope we haven't kept you hanging around.
21 **THE WITNESS:** No, no. You've been very prompt, actually.
22 Thank you.
23 **Questions from COUNSEL TO THE INQUIRY**
24 **MR SHARMA:** Lord Deighton, you've very helpfully provided to
25 the Inquiry a witness statement 30 pages in length. The

1 reference for it is INQ000536422. I wonder if you'd be
2 kind enough to confirm it's true to the best of your
3 knowledge and belief.
4 **A.** Yes.
5 **Q.** Lord Deighton, to begin, please, a little with your
6 biography. You were the chief executive of the Olympic
7 and Paralympic Games held in 2012.
8 **A.** Yes.
9 **Q.** You were a minister at HM Treasury between 2013
10 and 2015.
11 **A.** Yes.
12 **Q.** And in April of 2020, you became an adviser on PPE to
13 the Secretary of State for the Department of Health and
14 Social Care.
15 **A.** Yes, though initially when I was appointed, it was with
16 the specific objective of getting going on
17 UK manufacture of PPE, and it subsequently was expanded
18 to include the broader project.
19 **Q.** Thank you. And we'll come on to those two periods
20 during the course of your evidence.
21 To deal very briefly with your appointment, it was
22 in around April 2020 that you saw that the Covid crisis
23 was deepening and that you wished to volunteer to help;
24 is that right?
25 **A.** Yes.

1 **Q.** And so you approached officials in Number 10 to see if
2 you could assist and you were asked to support the
3 Secretary of State initially with the manufacture of PPE
4 in the United Kingdom?
5 **A.** Yes.
6 **Q.** The Inquiry has heard a lot of evidence about the fact
7 that global demand was soaring, the international
8 market, as you describe in your witness statement, was
9 breaking down, and that it was seen that domestic
10 production was a possible medium-term to long-term
11 solution to the current emergency; is that right?
12 **A.** Yes.
13 **Q.** Again, the Inquiry has heard much about the work of the
14 PPE Cell and those working extremely long hours; a real
15 fear that PPE would run out completely. Was that what
16 you saw when you joined the PPE team in terms of the
17 work ethic, the people you were working with? Does that
18 reflect your experience?
19 **A.** Yes. I mean, when I was appointed, what I decided to do
20 was to join in the team physically every day at the
21 National Health Service office in the Elephant & Castle
22 at Skipton House, because I just needed to absorb what
23 was going on as quickly as possible. And there
24 I worked, really for the next three months, in intense
25 collaboration with Emily Lawson, Jonathan Marron,

1 Phillip Prosser (from the army), to try to get our arms
2 around the problem.
3 I think my initial observation was that they had, in
4 very short order, put together a quite excellent crisis
5 response to an extraordinarily difficult situation. It
6 was very clear to me that anything that we'd had in
7 place before had completely broken down.
8 So, you know, I'm sure the Inquiry will look at the
9 resilience of what existed before. It was clearly not
10 sufficient to cope with the scale of the crisis.
11 It was also very clear that the sheer scale of
12 demand -- I mean, I think it's important to make this
13 point -- it wasn't just that we needed a bit more PPE;
14 we needed thousands of per cent extra PPE. So the
15 expansion in demand was -- it was more than exponential,
16 it was astronomical, and it was all descending on the
17 same constrained supply, mostly, it appeared, from
18 China, though it took us quite a long time to work out
19 where it was coming from because our historical
20 relationships had been confined to wholesalers.
21 And of course, the whole world was trying to deal
22 with the same problem of that enormous increase in
23 demand, and was competing ferociously for the very
24 finite amounts of supply available at that time.
25 **Q.** And so when you arrive, you see the team in the middle

1 of crisis management and, in the words of your own
2 witness statement, you describe good information as
3 being scarce, not only on the quantity of PPE which
4 was being purchased but also on the timing of the
5 delivery of the PPE to the United Kingdom, that was one
6 of the problems that you encountered?
7 **A.** Yes, indeed, because, of course, at this stage, all of
8 our incoming supply was being imported within the
9 chaotic supply and demand situation I just illustrated.
10 We really had no experience of that market in these
11 conditions, and so the team, every morning, would try
12 and work out who was about to run out, where the
13 potential supply to meet those shortages would come
14 from, and try to define exactly where the supply was in
15 that chain.
16 So, for example, we might spend a morning working
17 out how serious the traffic jam was getting a lorry to
18 Shanghai Airport cargo section. Those were the kinds of
19 things that the team was focused on, in a hand-to-mouth
20 arrangement at the very beginning when I got involved.
21 **Q.** I want to come on, please, to one of the first steps
22 which you took, which was to bring in external
23 experience and external advisers from the private
24 sector. But before I do so, to emphasise what you have
25 set out in your witness statement: that the team that

1 you were working with were working under extremely
2 stressful conditions, and you describe them as being --
3 having a clear sense of mission, and you described them
4 as being "the best of [British]".
5 And so what I'm asking you, about the bringing in of
6 external advisers, is not in any way to undermine the
7 efforts of the team that you saw working.
8 **A.** No, not at all. I mean, the effort I just described was
9 really working to bring in PPE from the international
10 markets. My mission initially was to get some real
11 traction with UK manufacturing, and the combined
12 resources of the government at that point were wholly
13 devoted to the international market, so there just was
14 no spare capacity to be able to make the significant
15 changes and jump-start we needed to really make
16 a difference for domestic manufacturing.
17 So I decided the only way we would be able to make
18 an impact was to bring in significant external resource,
19 with significant experience of what I would call
20 delivery. So people who could engage with manufacturers
21 and actually make it happen, as opposed to, you know,
22 constantly reviewing lists and triaging lists.
23 **Q.** You bring in a team from the private sector. Could you
24 take us through some of the expertise that those in the
25 team brought to the UK Make team and to the PPE Cell as

1 a whole?
2 **A.** Yes. When my appointment was announced, many people who
3 knew me asked if they could help. I mean, I think it
4 was the general sentiment in the country, there were
5 just many people who wanted to help but were unable to
6 find an appropriate channel. So my appointment enabled
7 some of those to say: look, can we help you?
8 As you mentioned, you know, I'd been -- I'd led the
9 Olympic Games and therefore I had many contacts who were
10 accustomed to working together under pressure and
11 delivering, and had a -- I would say, demonstrated to me
12 their capability, and there sort of -- we had a mutual
13 trust about being able to get difficult things done
14 so --
15 **Q.** Forgive me.
16 **A.** Yes.
17 **Q.** One of the aspects of your experience which you brought
18 to UK Make was that you had had experience, had you not,
19 of working both in the public sector and also in the
20 private sector? How do you think that assisted with
21 bringing others in from the private sector to work on UK
22 manufacturing?
23 **A.** What was essential to get things done was an ability to
24 integrate sort of the private sector capacity to drive,
25 innovate and deliver, with the public sector's

1 understanding of how you get things done in government.
2 So the key was not that one was any better than the
3 other; the key was being able to get them to really work
4 together.
5 The team I brought in, you know, many of them had
6 had experience either in the public sector or working
7 across in that way, so they understood very clearly what
8 they could do, which was to engage with the private
9 sector, with the manufacturers, to get them to the point
10 of delivery, but then they needed to work with those in
11 the public sector to get the support you needed there to
12 go through the contracting and regulatory approval
13 stages. So that integration was extremely important.
14 And the ultimate way the teams worked was very much
15 a combination of the private sector expertise that I
16 brought in, dovetailing with the public sector expertise
17 which was partly in place and partly brought in to help
18 me.
19 **Q.** Just those three words you used, "drive", "innovate",
20 "deliver".
21 Could you unpack those a little bit? What do you
22 mean by those in the private sector being unable to
23 drive? What do you mean by them being able to innovate?
24 And what do you mean by them being able to deliver?
25 What was it that was unique about those who were brought

1 in which wasn't available within the public sector as
2 you saw it?
3 **A.** I mean, I think we need to be careful about drawing too
4 extreme a contrast. The principal challenge to the
5 public sector resources, and indeed, I think you've
6 spoken to many of the procurement team from the Cabinet
7 Office who were working on the buy side, they too were
8 very experienced commercial operators, but they were
9 fully occupied, working with the overseas markets.
10 So the principal issue at the time with the public
11 sector was just a shortage of capacity, a degree of
12 exhaustion, having been throwing themselves at a very,
13 very difficult problem by that stage for over a month,
14 and so bringing in the new capacity with a very, very
15 clear focus, our mission was to get PPE manufactured in
16 the UK. So we removed the distractions of having to
17 respond to the thousands of different questions, whether
18 they were coming from the media or all the other, you
19 know, smaller offers that had been made, and I gave the
20 team a very clear mandate, which was to find the
21 manufacturers in each of the categories which could
22 produce the volumes we required quickly, to the quality
23 we needed, and at a competitive price. Very, very
24 simple mission.
25 **Q.** The second topic, if I may, following on from your

1 answer. One of the innovations which you brought to the
2 procurement and manufacture of PPE in the United Kingdom
3 was referred to by Mr Jarvis visit as "sprint and
4 category teams". Could you help us, please, with why
5 you thought that was the right response to the problems
6 of UK manufacture and what it was about sprint and
7 category teams in relation to supply chains of PPE that
8 made that the best approach?
9 **A.** I mean, firstly with respect to categories, I think it's
10 generally considered good practice to organise supply
11 chains along a product line basis, because --
12 **Q.** Allow me to interrupt. Why is that?
13 **A.** Well, because you can see the whole supply chain. So
14 giving one group of people the responsibility for
15 understanding every step of the way, and ultimately
16 being able to balance supply and demand, gave you a much
17 better sense of control over the process.
18 It also, given that we were essentially in start-up
19 mode, I needed the team to develop as quickly as
20 possible sufficient expertise to be able to be
21 effective, and category focus enabled them to get up to
22 speed very quickly.
23 I mean, and ultimately, I think, it was the right
24 way to go, because once, you know, by the middle of May,
25 towards the end of May, when I was trying to get my arms

1 with Emily and Jonathan around the whole effort, we
2 essentially reversed the category teams that I had into
3 the Buy Cell -- as I say, we moved from what I would
4 have described as the crisis response to a more
5 stabilised response, as we realised we were beginning to
6 get the right amount of supply in, both from manufacture
7 and from overseas, and that balancing supply and demand
8 was going to be an extremely important but difficult
9 challenge.
10 **Q.** So we have the balancing of supply and demand by the
11 approach that was adopted on the one hand, and the other
12 aspect of it was that the category teams could in effect
13 become experts in respect of each of the strands of PPE
14 with which they were charged with manufacturing and
15 procuring; is that right?
16 **A.** Yes, and it also, you know, when you look -- having
17 examined the landscape for that particular category, you
18 know, you could quickly determine: are there
19 manufacturers that already produce it? Are there
20 manufacturers overseas who know how to produce it who
21 could be persuaded to open a production line in the UK?
22 Were there manufacturers of an allied product which you
23 could help to adjust what they produced and how they
24 produced it, to create PPE? And then also, of course,
25 if you're dealing with a number of companies you can --

1 you're in a much better place to do price comparison,
2 quality comparison, speed comparison.
3 So for me, you know, it was clearly the right way to
4 proceed.
5 And then the other aspect which you brought up, of
6 course, were the sprints. We had no time, you know, as
7 we've already discussed, we were really too late. So we
8 couldn't spend weeks and weeks and weeks and weeks with
9 a market survey.
10 So essentially, I divided the categories into two
11 batches: half of them, batch 1, I gave the first four or
12 five days to the teams to survey the landscape, come up
13 with a plan, triage it down to the four or five
14 opportunities that we should pursue. Week 2, the other
15 half of the categories. And we then had a good enough
16 plan to begin to get into detailed discussion with those
17 manufacturers, and work out how to bring them through
18 the contract and technical approvals requirements.
19 **Q.** Thank you. And we'll come on to that in just a moment.
20 Your interface with those who were involved with the
21 manufacture of PPE, you took a different approach,
22 am I right, than the call to arms, in that you
23 identified and approached manufacturers, you selected
24 them -- we heard from Mr Jarvis -- on the basis of their
25 ability to scale up manufacturing at speed and at the

1 right price.
2 From the point of view of the manufacturers that you
3 were interacting with, what was it that they needed in
4 terms of support and assistance from the UK Government
5 and your team?
6 **A.** Yeah, I mean, the first thing they needed was our focus
7 and engagement so they knew we were serious about
8 getting this effort moving at speed, and with scale.
9 I think they then, you know, basically needed the
10 comfort that they were going to get a contract, because
11 that would enable them to bring -- in most cases, bring
12 their furloughed workforce back into the factory and, in
13 some cases, to invest in the machinery or move the
14 machinery around that they needed to make -- to produce
15 the equipment.
16 With respect to the -- you know, so we worked
17 extremely hard, with the help of -- I mean, you've heard
18 Mr Jarvis's evidence -- with the help of government
19 colleagues, to get as clear a definition as possible of
20 the product guidelines, because -- I mean, the
21 manufacturers were brilliant in their response. I mean,
22 most of the innovation, the urgency, the delivery came
23 from them, once we put them in a position to be able to
24 respond. So I don't know how clearly that's come out in
25 the evidence to the Inquiry so far, but I really do want

1 to salute the response we got from British industry.
2 Once we made it possible for them to contribute, they
3 were outstanding in their response.
4 **Q.** Could I just interrupt you there just for a moment. So
5 one of the areas in which manufacturers needed
6 assistance was in terms of financial assistance; is that
7 right? Support from the government?
8 **A.** Well, the principal way they needed financial assistance
9 was the assurance of the contract. So getting paid was
10 actually the principal thing that made them happy to get
11 up and get going. And then there were other bits of
12 help that we gave them. But they were extremely
13 creative about solving the problems themselves, because
14 they were motivated both to put volume through their own
15 operations, which were suffering at the time because we
16 were in the midst of Covid, bring their workforce back,
17 as well as a very strong motivation -- which I sensed
18 everywhere I got involved in this project -- to help.
19 So those -- they were extremely motivated, and we
20 worked with the technical -- the regulators, essentially
21 to make sure we had the clearest possible product
22 guidelines, because once there was clear definition,
23 these manufacturers were in the main wholly capable of
24 producing to a well defined product definition, you
25 know, right down to creating templates for them that

1 they simply needed to copy.
2 You then need to take them thorough a testing
3 process, which, you know, had its challenges. I mean,
4 if we step back and just look at the whole environment
5 for producing and getting to the workforce PPE --
6 **Q.** Lord Deighton, forgive me, I just wanted to bring you
7 back to this subject and then we'll come on to the
8 regulatory approvals and the testing houses.
9 **A.** Okay.
10 **Q.** Just to come back to what the manufacturers needed, if
11 not financial support, we've heard from Mr Jarvis visit
12 that some of the assistance they required was in order
13 to re-tool and provide blueprints and specifications for
14 the sorts of equipment that they needed to produce. How
15 did your team assist them in that process?
16 **A.** The team I brought in was working with the
17 manufacturers. They would essentially handhold each of
18 their client manufacturers through the process with the
19 different parts of the regulators and the government to
20 meet each of, I think, what elsewhere has been defined
21 as the eight steps of due diligence. But of course when
22 you're dealing with a domestic UK manufacturer, many of
23 those are extremely straightforward. You know who
24 you're dealing with, you know what their financial
25 condition is. So most of the approval issues become

1 those around regulation and safety. And that's where
2 a lot of the work was focused.
3 And we would help bring together those people in
4 government who understood what was required, and
5 essentially provide the interpretation between what the
6 company needed to know, what the regulators needed to
7 see, and ensure that that was satisfactorily resolved at
8 pace.
9 **Q.** You've referred to the eight-stage process. I wonder if
10 we could go to some of the written evidence in your
11 witness statement.
12 It's at INQ000536422.
13 This is page 7 of your statement and paragraph 19.
14 **A.** Yeah.
15 **Q.** I just want to take you thorough a few paragraphs of
16 your statement about the eight-stage process and how you
17 assisted manufacturers through it. You describe here
18 that:
19 "Our team had managed to streamline the entire
20 end-to-end process of design through to manufacture,
21 including procurement processes and governance
22 approvals, to ensure [that] new domestic PPE supplies
23 were rapidly approved in a robust and legally compliant
24 way."
25 We'll come back on to the issues with safety

1 regulations in a moment.
2 "Key to this streamlining was the appointment of
3 category leads ..."
4 Which you've covered. And then:
5 "Rather than changing the regulatory procedures
6 which were ... consistent and thorough, companies were
7 better guided through each step by having
8 one representative and point of contact throughout ..."
9 And this is the area I'd like you to focus on,
10 please:
11 "... therefore avoiding the cumbersome handover
12 process between each stage of the approvals process."
13 Could you help us, please, with, first of all, what
14 is it you mean by streamlining the entire end-to-end
15 process and, secondly, what is it that you mean by the
16 cumbersome end-to-end process?
17 **A.** Yeah, I think the key thing was that we had a relatively
18 small and focused group of significant manufacturers,
19 each handheld by a member of the team. So nobody was
20 left -- nobody from the outside was left really having
21 to negotiate each part of our own process. They were
22 taken through that. So the streamlining really was the
23 way in which it was handled to take them through there.
24 We'll talk about some of the regulatory things later but
25 there was some streamlining there, moving sequential

1 processes on to parallel processes.
2 I think I've already referred to the fact that by
3 this stage, when we had a supplier going through the
4 contractual stage, the category team would also have
5 included somebody from the government side who is very
6 familiar with the contractual process, so again,
7 integration was absolutely key.
8 **Q.** Could I just interrupt you there. You referred to
9 moving from a sequential process to a parallel process.
10 What do you mean by that?
11 **A.** Well, the nature particularly of product approvals was,
12 you know, you do one thing, then you go to the next
13 thing. And in certain cases, when we looked at it,
14 there was no reason for one to follow on from the other.
15 You could do two at the same time. In peacetime, in
16 normal time, of course, the whole point about safety is
17 that it's a slow and highly deliberate process. We
18 still needed deliberate but we didn't need slow. So we
19 could parallel up the normal processes that they would
20 go through, and then you simply have to deal with
21 capacity issues. And as you mention, testing houses was
22 one of the constraints we had.
23 So there was some attempt, again, with the help of
24 our friends in BEIS -- and you heard from the excellent
25 Mr Jarvis -- to increase capacity domestically and then

1 to add shift work on that capacity so you could work
2 three shifts a day rather than the customary one.
3 So those were the kinds of things that, you know,
4 one does in a crisis to expand capacity and to speed
5 things up.
6 **Q.** So what you wanted, from your point of view, was a move
7 from the step-by-step process to a process in which
8 multiple parts of that process could be completed at the
9 same time. And is that essentially what you and your
10 team were leading the manufacturers through?
11 **A.** Yes, and it's, as I think you've already suggested, much
12 easier to accomplish that with domestic manufacturers
13 because they're there, you can deal with them, it's
14 face-to-face, you can see what they're producing, the
15 prototypes. Whereas my colleagues in the Buy Cell,
16 trying to work out who was a manufacturer in China or
17 somewhere else, who were -- where the wholesaler was,
18 whether the wholesaler was ordering from another
19 wholesaler with lots of companies that seemed to have
20 been recently set up, with limited histories, each one
21 of those steps could really be very, very difficult for
22 them to confirm. Which is why, in that case, it was
23 much more likely you'd get lost in a process, because
24 many of the steps weren't being properly fulfilled and
25 people were being sensibly cautious about charging

1 ahead.
2 **Q.** Could you help us with this, please: you've described
3 the manufacturers that you're dealing with as having
4 been experienced previously in manufacturing, perhaps
5 not necessarily in PPE or even related products. Why
6 was it that they were finding such difficulty in
7 navigating this process that DHSC had established?
8 **A.** They weren't. The manufacturers in the main had not got
9 to that point. I think once we'd established our
10 focused, fully staffed, ready-to-go Make effort, the
11 handholding through the process worked well. So they --
12 it wasn't that they were stuck. I think, when I'm
13 describing some of the challenges in the process,
14 I think that's what our colleagues in the Buy Cell,
15 looking at the international market, were experiencing,
16 for all the reasons I just laid out.
17 **Q.** You describe further in your statement -- and we don't
18 need to bring it up -- not only as cumbersome but that
19 you needed to move to a more efficient approach to
20 oversight of supply and demand.
21 In fact, maybe we should bring this paragraph up.
22 It's INQ000536422. It's page 27 of your statement and
23 paragraph 84.
24 Thank you.
25 You said as you have set out in more detail above,

1 the restructuring of the teams you have described
2 resulted in a more efficient oversight of supply and
3 demand for each category of PPE.
4 Is that what you were referring to when you were
5 describing having organised the teams into categories
6 and therefore being able to better predict what was
7 being supplied and what was being demanded?
8 **A.** Well, certainly one component of establishing better
9 control over the crisis situation, in my view, was
10 having a category-structured organisation. But that was
11 really only one component. You know, the real -- once
12 I felt that we had established an excellent Make effort
13 where I had very strong leaders managing, sort of, the
14 day-to-day process of contracting and delivery, I moved
15 over to look at some of the problems in the broader
16 effort, working very closely with Emily Lawson and
17 Jonathan in particular.
18 And the real issue was establishing the facts.
19 I mean ultimately, one of the reasons I was there was to
20 assure the Prime Minister, who was extremely focused on
21 this topic, that we had enough PPE. Very simple test,
22 because then we -- that had implications for lockdown,
23 and it meant that resources could move on to other
24 priorities like the vaccine, like test and trace. So
25 the Prime Minister was insistent that I could

1 demonstrate to him that we had enough.
2 You know, I was there because we had a relationship
3 given how I'd worked on the Olympics when the
4 Prime Minister was the mayor.
5 And in order to do that, we needed better facts. We
6 needed to understand how much PPE we already had; we
7 needed to understand how much was being used, which
8 required an understanding of how many people were going
9 to get Covid, what kind of interactions the medical
10 staff would have with the patients. I then also needed
11 to understand every step in the supply chain to
12 understand where the orders that had been placed were,
13 and whether they -- and when they were going to show up,
14 and what sort of state they were going to be in when
15 they got there.
16 So we had to create a model out of that to determine
17 where we would have shortages and where we would have
18 surplus to create what, in the supply chain terminology,
19 would be called a demand signal. So getting better and
20 better information to make that demand signal more and
21 more precise.
22 So sort of the middle to the end of May, I worked
23 with the team to try to get a grip of that, and I think
24 by 21 May, third week of May, I was able, with the team,
25 to say to the Prime Minister "We have it covered for

1 both the short and the medium term" because, of course,
2 there was a lot of focus on being prepared for the
3 anticipated second wave in the coming winter.
4 **Q.** Lord Deighton, that is, if I may describe it as the data
5 problem that the Inquiry has heard some evidence about,
6 the effect of there being a model for the amount of PPE
7 which was anticipated would be required during the
8 pandemic, and the effect of delays in delivery, for
9 example, of equipment that was being bought from
10 overseas, the breakdown of logistics and supply chains
11 into the United Kingdom; is that right?
12 **A.** Yes. So every part of that dataset to create an
13 accurate model had significant elements of uncertainty,
14 and you've described it well on the supply chain side
15 because it was very difficult to know, given the chaotic
16 state of things, exactly when things would turn up and
17 whether they would be safe to release to the front line.
18 And on the demand side, it was -- you know, good
19 demand modelling or accurate demand modelling is at its
20 best when it can be supported by the most recent and
21 equivalent experience. The problem here was nobody had
22 any experience of Covid, how it would spread. Nobody
23 had any experience of the utilisation of PPE during
24 Covid. Add therefore, trying to estimate into the
25 future how much of this you needed was heavily

1 assumption based and therefore subject to considerable
2 error.
3 Q. Thank you.
4 Lord Deighton, we're going to come back on to that
5 in a moment.
6 A. Yeah.
7 Q. If I can bring you back, please, to the issue of
8 regulation and technical specifications --
9 A. Yeah.
10 Q. -- and the persistent, if I may describe it in those
11 terms, problems of complexity of regulation and getting
12 equipment from manufacturers to the front line. Now, of
13 course, no one, not least anyone in this Inquiry,
14 suggests that even in an emergency, that regulation
15 about safety is something which ought to be compromised
16 with.
17 But from your perspective, and dealing with the
18 manufacturers at the time, was there complexity in
19 regulation, difficulty in dealing with the regulators,
20 and trying to navigate that regulatory landscape?
21 A. Yes. I mean, I think with any safety system the issue
22 is never keeping the bad stuff out; the issue is always
23 getting the good stuff through quickly. Right? That's
24 always the challenge. And I'm sure every witness has
25 told you that, actually, the processes to ensure that

1 nothing unsafe got to the front line were never
2 compromised, and that actually gave me a certain amount
3 of comfort as I pushed for speed. So, for me, it's
4 actually comforting that there was a degree of pushback
5 to make sure that the product was safe.
6 So this is not an unhealthy tension, in many
7 respects.
8 Q. And so just to be clear, when you're talking in your
9 witness statement and your evidence to the Inquiry today
10 about streamlining and speeding up processes, you're not
11 describing, are you, compromising on the safety
12 standards that either UK Make or the PPE Cell in general
13 was approaching the problem with?
14 A. No, we'd -- the regulators -- I mean, we would take to
15 the regulators a case of something we wanted to do
16 slightly differently, or slightly more quickly, and
17 essentially put to them the question: what does it take
18 for you to be clear that this will be safe?
19 And they were prepared to be flexible in saying:
20 "Well, you're right, we don't really need to do that
21 thing that we would have traditionally done. If you do
22 this, that'll be fine."
23 So it was a case-by-case, pragmatic,
24 working-together crisis approach to helping everything
25 through, making sure that the safety concerns were still

1 appropriately considered.
2 Q. Could we have a look, please, at perhaps some practical
3 examples as to how this played out.
4 INQ000477711.
5 These are the minutes of what was called the
6 Regulatory Co-ordination Cell, which brought together
7 entities such as the Health and Safety Executive, the
8 MHRA, and the OPSS, whose witness evidence we've heard
9 from Mr Graham Russell.
10 This was on 5 May 2020, not a meeting which you were
11 present at but at which reference to you is made.
12 Under "Public face coverings", please, it says under
13 here that:
14 "[The] HSE [is] holding the position [they're
15 talking about public face coverings] are not PPE, and
16 [the] MHRA are confirming that these do not meet the
17 definition of a medical device."
18 And then there being other issues.
19 And then if we can come down to the decision making
20 committee, DMC, beneath. There's a reference there to
21 an apparent disconnect between you, Lord Deighton, and
22 HSE on pre-approved designs and specifications, and
23 that -- is this is an example of the sort of pragmatic
24 approach which you're describing? What was happening
25 here, from the best of your memory?

1 A. My memory doesn't connect to this specifically. It
2 sounds very "Yes Minister", doesn't it? I was
3 apparently disconnected. I think I would have been
4 driving hard to push things through, and this was all
5 part of the negotiation, to get us moving at speed.
6 And 5 May was pretty early in my tenure there, so
7 this would have been at the beginning of trying to find
8 the right equilibrium between push and accept, the
9 tension I described earlier.
10 Q. You describe in your witness statement -- we don't need
11 to bring it up, but it's at paragraph 83 -- that you
12 believe this largely related to the need of
13 clarification of technical specifications and ensuring
14 clear communication to the manufacturers, and that you
15 saw the role of your team was to deal with any
16 uncertainty that the manufacturers would have in
17 relation to the guidance and the specifications of PPE.
18 A. Yes.
19 Q. So is it right that, rather than seeking to solve the
20 underlying problem of the complexity of the regulators
21 and the regulation, that you and your team saw
22 yourselves as problem solvers, to make sure just that
23 the PPE and those manufacturing it could navigate their
24 way through the system?
25 A. Yes. Let me give you an example. So aprons. There

1 were no manufacturers of aprons in the UK, but aprons
2 are made of plastic, and there are people who make
3 plastic things. So we found out who the best
4 manufacturers of plastic bags were, and the essential
5 difference between a plastic bag and an apron is that
6 you have to cut three more holes in the bag to put your
7 arms and head through. Therefore, what we had to do
8 with the apron manufacturers were to help them install
9 cutting machines for the final end of the process, and
10 then to help them determine how to package the aprons,
11 because the health service likes to pull them off a reel
12 rather than take them out of a box.
13 The other thing that matters is the weight. People
14 don't like their aprons too light because they fly up.
15 They don't likely them too heavy because they're
16 constraining.
17 So, in the case of aprons, getting the weight and
18 the distribution mechanism right were the key things the
19 manufacturer had to understand.
20 In the case of masks, I think the initial contract
21 we worked on was with Honeywell. Honeywell clearly knew
22 how to make masks. They were making them all around the
23 world. And the proposition with them is that they
24 opened a production line for masks at their factory in
25 Motherwell. And so they really just needed to

1 understand what do the British standards require.
2 And so, in each case, you could see that there were
3 particular things that specifically mattered for getting
4 that order to the front line.
5 **Q.** The Inquiry received evidence from Mr Russell from the
6 OPSS. And he was asked as to whether the regulatory
7 landscape which the manufacturers that you were working
8 with was fragmented and complex. And his evidence was
9 that, from the regulators' point of view, they had to
10 take into account the fact that the equipment that was
11 being manufactured was going to be used by different
12 people in different places and for different purposes,
13 and that where regulation was there to mitigate any
14 potential risk, that you would always have to find
15 a dividing line between different regulators and
16 different regulations?
17 Was that a problem for the manufacturers who, tell
18 me if I'm wrong, had in their mind that what they needed
19 to manufacture was PPE to be used in a health emergency
20 and to manufacture it as quickly as possible? The
21 manufacturers were results orientated; they were looking
22 at this from the point of view of: how do we make the
23 PPE for the emergency at hand?
24 **A.** Yeah. I mean, I think the -- I mean, you're right, in
25 a situation where you're trying to move at speed, having

1 a fragmented environment to operate against is not
2 helpful. I think what the regulators usefully did was
3 to come together and to act in a coordinated way. There
4 was some discussion as to whether lead regulator status
5 should be given to one so we could just get a "Yes" or
6 "No" from one. But I think as Mr Russell is implying
7 there, that wasn't something that they felt would work,
8 given the different responsibilities they had. But
9 I think, given the short notice we had, given the nature
10 of the crisis, by the -- because we were giving them the
11 right focus questions, I wasn't unhappy with their speed
12 and flexibility of response.
13 **Q.** Would it have made your job, and the job of the
14 manufacturers, easier for there to have been a lead
15 regulator or for there to have been a single, to use
16 your words, streamlined set of regulations, still with
17 an eye to safety at all times so that in the case of
18 an emergency, if a manufacturer needed specifications,
19 blueprints, and so on, they had a single place to look
20 and a single regulator through which to get that which
21 they were manufacturing through the system?
22 **A.** Yeah, I mean, I think yeah, by definition that has to be
23 quicker. And what you're really doing there, if these
24 regulators have different responsibilities, is you're
25 asking the lead regulator to take responsibility for

1 getting agreement from the other regulators. And given
2 that they speak the same language, and broadly have the
3 same objective, you would expect them to do that better
4 than for non-experts to have to negotiate with each of
5 them bilaterally.
6 **Q.** Thank you, Lord Deighton.
7 Just moving on to another topic, please: to explore
8 with you what you consider were the root causes of the
9 UK's over-purchasing of PPE. You've touched on some of
10 the reasons for it a little earlier: the data problem,
11 the inability to predict that which was in demand and
12 what was going to be supplied. Could you provide your
13 reflections on other reasons that you think that the UK
14 had over-purchased PPE.
15 **A.** Well, of course, if you look at the documents that the
16 team produced at the time, we weren't consciously
17 overbuying, we were buying to the modelled demand. So
18 the answer to the question sits in an analysis of why
19 those estimates or modelling proved incorrect.
20 And I think I've referred to some of those already.
21 It has a lot to do with modelled demand being in excess
22 of actual demand. Partly that will be because we were
23 conservative in the modelled demand. Partly it will be
24 because people were rationing their actual utilisation.
25 And so the difference diverged. And just think about

1 it, if you use a mask twice rather than once, you've
2 halved the number of masks you need. It makes an
3 enormous difference.
4 I think by the beginning of June, this was one of
5 the advantages of the category approach, my --
6 I remember vividly my category lead on eye protection
7 saying, "Ignore the model, I can just see what's going
8 to start piling up in the warehouse. Let's stop
9 ordering."
10 Which we did. But the model would have suggested
11 you kept ordering. So --
12 **Q.** And I think that's what you refer to in your written
13 evidence as "commonsense adjustments" to the PPE that
14 was being purchased and manufactured?
15 **A.** Once you had enough visibility to apply common sense,
16 yes.
17 **Q.** One of the critical problems that Mr Marron explained to
18 the Inquiry about predicting the supply of PPE during
19 the pandemic were the problems caused by the lead times
20 in respect of equipment that was arriving from overseas.
21 And then secondly, as to whether what was arriving from
22 overseas markets would be of suitable quality. There
23 was another unknown factor in that. It was difficult,
24 if not impossible, to conduct inspections at remote
25 locations.

1 Are there any advantages from the UK Make and the
2 UK manufacture in respect of that problem?
3 **A.** Yeah, I mean, in every respect, if you have access to
4 competitive, high-quality domestic manufacturing, it
5 takes all of the challenges around the uncertainty of
6 deliveries from overseas in a difficult market
7 environment away from you, right? You have immediate
8 control and visibility on every aspect.
9 I mean, we could see the pallets piling up of -- at
10 the end of the production line. We could -- we had
11 control over the lorry to take it to the warehouse and
12 then to distribute it. So it both would significantly
13 shorten and give you total control and visibility over
14 the supply line. Clearly the international buying we
15 were doing didn't really have any of that.
16 **Q.** And what about with respect to coordination, domestic
17 manufacturing? How can coordination be improved by the
18 fact that items are being manufactured in this country
19 as opposed to being purchased from overseas?
20 Forgive me, let me explain that. What I mean by
21 "coordination", I mean coordination with other aspects
22 of government policy. So other -- for example, the
23 rollout of the vaccine and the effect that would have on
24 the demand for PPE on lockdowns, on hospitalisations.
25 How is coordination made better by reliance on domestic

1 manufacturing?
2 **A.** Well, it simply gives you control. It's as simple as
3 that. Your destiny is much more in your own hands. The
4 uncertainty is diminished by a significant number of
5 factors, as we've discussed.
6 **Q.** And finally, if I may, turn to the issue of timing. The
7 UK Make Programme really only started getting going in
8 April with delivery of the first items of PPE, according
9 to Mr Marron, just 25 days after it was established.
10 Now, of course, not -- this is with the benefit of
11 hindsight, but with foresight and planning about scaling
12 up the UK domestic manufacture of PPE, what effect would
13 that have had on PPE procurement during the pandemic?
14 **A.** Well, with foresight and planning you probably would
15 have pressed that button in February. I think -- and,
16 you know, I've agonised over this -- I think, even in
17 the crisis we were in, the effort I was able to start at
18 the end of April, I see no reason why that couldn't have
19 started at the end of March, had we got ourselves
20 organised and focused that way.
21 **Q.** And as part of that foresight and planning, you provide
22 in your written witness statement a number of
23 reflections, including the identification early, perhaps
24 pre the next pandemic, of strategic manufacturers in the
25 UK; is that right?

1 **A.** Yes.
2 **Q.** And the other observation that you make, in addition to
3 diversification of supply from other jurisdictions, is
4 to keep an eye on access to raw materials. Can you
5 explain why that is important to domestic manufacture.
6 **A.** Yeah, indeed, because clearly you don't solve the
7 problems we were discussing by simply having the
8 manufacturing capacity if you're exposed to the
9 international supply of some of the raw materials. So
10 you have to trace your domestic supply chain back
11 through, you know, to every single component part, to
12 make sure that you have access to that, if it's going to
13 be effective.
14 **Q.** Thank you, Lord Deighton.
15 Just before I close, I wonder, are there any other
16 further reflections that you have in relation to your
17 experience of the PPE Cell and the UK manufacture of
18 PPE?
19 **A.** I mean, I'll just reinforce I think, you know, a couple
20 of the points that I've already made. I mean, firstly,
21 the problem was our lack of preparation. Those who came
22 in to deal with the crisis were heroes. And nobody
23 could have done any better.
24 When I was brought in by the Secretary of State and
25 the Prime Minister, one of the things they really wanted

1 me to do was to check whether the people working on it
2 were okay, because I arrived in a situation where the
3 country was hurting, where the government was seen not
4 to have a grip, and we had all these offers of PPE, yet
5 the front line was running out.
6 And so when I went down to Skipton House on that
7 first morning, I was mostly trying to work out whether
8 the team there was up to it. And they were 100 per cent
9 up to it. So I think it is important for the Inquiry to
10 understand that I think the work done in the heat of
11 battle was outstanding.
12 And I think the other thing I would just call out --
13 and I have, I think, already referred to this -- I mean,
14 the response of the manufacturers to get this done,
15 fast, and, you know, delivering stuff that was
16 competitive, that passed the safety test, that was used,
17 was outstanding. And I think we should acknowledge that
18 in -- when we reflect on this crisis.
19 **MR SHARMA:** Thank you very much, Lord Deighton.
20 My Lady, I don't have any further questions but
21 there are -- (overspeaking) --
22 **LADY HALLETT:** I think there are some questions. I think
23 Ms Morris is going first. She sits just there.
24 **Questions by MS MORRIS KC**
25 **MS MORRIS:** Thank you, my Lady.

1 Lord Deighton, I ask questions on behalf of the
2 Covid Bereaved Families for Justice UK. I am first
3 going to highlight four pieces of evidence to you, if
4 I may, and then ask you two questions about them.
5 The first piece of evidence is an email -- that's
6 INQ000494747 -- on 30 April 2020.
7 This is an email between yourself and Lord Feldman,
8 and it's the email at 11.54, please, just towards the
9 top there.
10 Thank you. It says there:
11 "Dear Paul" -- that's yourself.
12 "A connection with a UK based visor manufacturer
13 struggling (for a change) to get any traction with the
14 NHS. Introduced by the wonderful Stuart Marks ..."
15 That's Lord Marks of Hale, is that correct?
16 **A.** (No audible answer).
17 **Q.** "... the Northern Treasurer of the Party and tech
18 entrepreneur, so a really trusted source. Seems to sit
19 in your remit.
20 "Warmest best wishes.
21 "Andrew."
22 And that's Lord Feldman.
23 The next document, please, INQ00494748. This is an
24 email now dated 27 May 2020. It's the email at 18.55 at
25 the top, thank you. This is Lord Marks emailing you

1 directly this time:
2 "Hi Paul.
3 "Hope you are ... well. Wondering if you can be of
4 any help with this. As mentioned in earlier emails, the
5 company are desperately trying to hold on to staff to
6 avoid furlough but have been waiting for confirmation on
7 whether the company has been selected. The CEO at Vogue
8 [that's the company that Jonathan Salem was
9 representing] has been speaking to [X] who has been
10 helpful but is unable to prioritise the case and says he
11 is waiting for a closing team member to pick this up.
12 [They] believe they have a competitively priced UK
13 manufactured product and have been approved to supply.
14 "Is there anything you can do, [the] CEO really
15 needs to make some business decisions.
16 "Best regards, Stuart."
17 The CEO there is a Mr Jonathan Salem and the Inquiry
18 has a statement from Lord Marks who confirms that
19 Mr Salem was a friend of a friend, and the number that
20 Mr Salem had founded represented the paper drinking
21 straw company.
22 It's since been reported by the Good Law Project
23 that the PPE supplied by the paper drinking straw
24 company was entirely unfit for use and that every single
25 one of the surgical masks that they supplied was unfit

1 for use.
2 So my questions are this: why does being introduced
3 by a Conservative Party treasurer, that's Lord Marks,
4 make a potential supplier "a really trusted source", in
5 Lord Feldman's view?
6 **A.** Well, ask Lord Feldman.
7 **Q.** Did you agree it was a really trusted source?
8 **A.** Indifferent.
9 **Q.** Following these emails, did you take any steps yourself
10 to assist this company to secure a contract for PPE
11 manufacture?
12 **A.** Well, I think if look at those two, the first email was
13 the end of April.
14 **Q.** Yes.
15 **A.** The next one was the end of May. You can see how much
16 urgency I attached to that introduction. So no.
17 **Q.** Did they, to your knowledge, have their hands held in
18 the way you described in your evidence this morning?
19 **A.** No.
20 **Q.** Next set of questions, please. And this is another
21 email chain.
22 INQ000494749, 17 May.
23 This is again an email from Lord Marks, this time
24 forwarding on a message from Haraldur Agustsson.
25 Lord Marks says at the top there:

1 "Dear Paul,
2 "I would be grateful if you could take a look at
3 this, Haraldur is a generous and loyal supporter of
4 the Party ..."
5 And you respond, just a bit further up, saying,
6 "Will do".
7 So, in respect of this referral, what did you do in
8 response to this email? Did you do anything to assist
9 this company to secure a contract?
10 **A.** Well, if you read this email, he is essentially
11 complaining that we have favoured a series of other mask
12 manufacturers. Alpha Solway -- and he actually has
13 a point, Alpha Solway was a serious mask manufacturer,
14 so we should have considered him seriously, but
15 unfortunately he didn't make the initial triage.
16 **Q.** So now he's coming to you and the information you've got
17 is he's a generous and loyal supporter of the party?
18 **A.** Irrelevant. That is irrelevant to me.
19 **Q.** Okay. But do you accept that these sorts of
20 introductions and direct requests for personal
21 assistance from -- to you amounts to a request for
22 preferential treatment by political connection?
23 **A.** I think if you'd have been -- I was bombarded by
24 different parties trying to help, is how they would have
25 described their situation. Personally, I find it odd

1 and slightly off-putting when the introduction takes
2 that form. I've been in a number of jobs where I have
3 power, and therefore people seek to influence me.
4 Frankly, when they do that, it's unhelpful, rather than
5 helpful.
6 **Q.** Did you consider whether these raised conflicts of
7 interest or due diligence issues?
8 **A.** Yeah, I mean, I think that had I pursued it on any
9 favoured basis, I absolutely recognise that. But
10 I didn't.
11 I think as you've seen from the way I triaged all
12 the Make contracts, we had one set of criteria, which
13 were about the ability of the company to deliver the
14 right product fast, at scale, at a good price.
15 **Q.** So, from your answer a moment ago, do you consider these
16 approaches to be at all improper at the time? You said
17 you were bombarded.
18 **A.** I think it was the nature of where we were that the
19 government and the country had asked for help. We
20 clearly didn't have enough PPE. People who thought they
21 could help with that were trying by every means they
22 could to advance that. I agree, you have to be
23 thoughtful about how you then prioritise and allocate
24 efforts. I don't think I could have been clearer about
25 how I did that.

1 **Q.** As part of that thoughtfulness, did you inform the Civil
2 Service procurement officials about the source and
3 nature of these introductions in advance of the offers
4 proceeding through to the opportunities stage?
5 **A.** I wasn't part of that process.
6 **MS MORRIS:** Thank you.
7 Thank you, my Lady.
8 **LADY HALLETT:** Thank you, Ms Morris.
9 Next I think it's Mr Stanton, who is right over
10 there.
11 **Questions by MR STANTON**
12 **MR STANTON:** Good afternoon, Lord Deighton.
13 **A.** Good afternoon.
14 **Q.** I ask questions on behalf of the British Medical
15 Association.
16 At paragraph 21 of your statement you refer to your
17 efforts to get a clear picture of PPE supply and demand,
18 and you make reference to a piece of work carried out by
19 McKinsey and Company in conjunction with DHSC. I'd just
20 like to take you to a few points in that particular
21 demand report.
22 It's at INQ000339131, page 3, in the first instance,
23 please.
24 And Lord Deighton, at the top line of the document
25 before you, you can see it concerns the issue of FFP3

1 respirators, and the concerns identified in the second
2 column are that demand will burn down existing stock,
3 and there's also a concern identified that the forecast
4 supply was inconsequential versus demand.
5 And then in the final column, "Actions", there are
6 some proposed actions The first bullet point, "Increase
7 acquisition". The last-third bullet point "Approve FFP2
8 masks as a substitute".
9 But it's the second bullet point that I'd like to
10 bring to your particular attention, please. "Reduce
11 demand with policy".
12 And you'll find a second reference to this proposal
13 at page 5 of the document, which hopefully you'll have
14 before you. Yes. The shaded box on the left, Lord
15 Deighton, indicates a proposal to ensure that there is
16 sufficient PPE in the right place at the right time for
17 all staff. And then bottom line, again, we have
18 proposals, one of which is optimise usage, and
19 a proposed action to revise the guidance, eg PHE, Public
20 Health England.
21 And then the final reference, please, Lord Deighton,
22 is page 10. And here hopefully you should see --
23 **A.** Yeah.
24 **Q.** -- the guidance or policy on use of PPE. The first box
25 deals with high-risk procedures and settings, and

1 indicates the use of FFP3 respirators for those
2 purposes. And then in the second box, other settings,
3 we see here, even healthcare workers who are engaged in
4 close personal care of patients with Covid-19 don't get
5 an FFP3 respirator; they have a fluid repellent face
6 mask.
7 My question, Lord Deighton, given your role in
8 leading the UK's domestic manufacturing of PPE, is: were
9 you aware of the recommended action to manage and reduce
10 demand for FFP3 masks through policy rather than by
11 prioritising procurement and manufacturing?
12 **A.** This document I think was 29 March. I joined a month
13 later. The situation had -- was just completely
14 different by that point. So these were clearly, you
15 know, much earlier efforts to deal with the chaos
16 that -- this was the high point of panic and
17 uncertainty. So that's the environment. So by the
18 time, you know, four weeks was a lifetime during that
19 period. So this document is, you know, just completely
20 out of date for the situation I was confronting.
21 **Q.** So at the time you took up your post, these issues were
22 not issues that were brought to your attention, these --
23 **A.** We were not trying to constrain demand. We were just
24 trying to get every single piece of PPE we could,
25 because we understood the anxiety, the fear, on the

1 front line. Absolutely.
2 **MR STANTON:** Thank you, Lord Deighton.
3 My Lady, thank you.
4 **LADY HALLETT:** Thank you, Mr Stanton.
5 Is it -- it's Mr Dayle.
6 **Questions from MR DAYLE**
7 **MR DAYLE:** It is.
8 Thank you, my Lady.
9 Lord Deighton, I ask questions on behalf of FEMHO,
10 the Federation of Ethnic Minority Healthcare
11 Organisations, and I have one short topic about the
12 structures that existed for procurement decisions. And
13 my question is this: what frameworks or oversight
14 mechanisms were in place with PPE Make to ensure
15 procurement decisions met the needs of all healthcare
16 workers, but specifically those from ethical minority
17 backgrounds?
18 **A.** The supply side was really responding to the guidelines
19 and requirements that came from the National Health
20 Service. And so the procurement was, certainly in the
21 early stages, just trying to get more equipment.
22 What became apparent -- and it probably became
23 apparent to me mid to the end of May -- was that there
24 was some size issues, some shortage of very small and
25 very large sizes, and then that became particularly

1 focused on FFP3 masks, which are the tight-fitting heavy
2 filtration masks which are uncomfortable to wear for
3 a long period if they don't fit properly. And, of
4 course, if they don't fit properly there's a risk of
5 exposure to the virus.
6 And once we had established that that was a problem,
7 there was a significant effort that went into assuring
8 we had a range of supply and very proactive fit testing
9 so people could always have the mask that they wanted or
10 needed.
11 I acknowledge that there must have been periods in
12 the height of the pandemic when workers felt unsafe.
13 I can certainly see from the subsequent evidence that
14 was particularly the case with certain minorities.
15 I wish we could have dealt with that. Future planning
16 needs to make sure we can, but, as I explained to you,
17 for us, it was a size issue and when we saw it, we
18 worked very hard to address it.
19 **MR DAYLE:** Thank you, my Lady.
20 Thank you, Lord Deighton.
21 **LADY HALLETT:** Thank you, Mr Dayle.
22 That completes the questions we have for you,
23 Lord Deighton, I am extremely grateful to you.
24 Since taking over this job I've heard many dreadful
25 stories, as you will imagine, but I've also heard some

1 positive ones. And if I may say so, the team effort
2 that you led to obtain vast quantities of PPE in
3 extraordinary circumstances was one of the positive
4 stories, so thank you very much for all that you did.
5 Thank you also for highlighting the role of the
6 manufacturers. It's not something I've heard too much
7 about yet, and I think it was important to mark, so
8 thank you for your help to the Inquiry as well.
9 **THE WITNESS:** Thank you, my Lady.
10 **LADY HALLETT:** Very well, I shall return at 1.45.
11 **(12.47 pm)**
12 **(The Short Adjournment)**
13 **(1.45 pm)**
14 **LADY HALLETT:** Mr Wald.
15 **MR WALD:** My Lady, our next witness today is Lord Agnew.
16 **LORD AGNEW (sworn)**
17 **Questions from LEAD COUNSEL TO THE INQUIRY FOR MODULE 5**
18 **LADY HALLETT:** I hope we got our timings right so you
19 haven't been twiddling your thumbs waiting to come on.
20 **THE WITNESS:** No, thank you very much. I've been well
21 looked after.
22 **MR WALD:** Please state your full name for the Inquiry.
23 **A.** Theodore Thomas More Agnew.
24 **Q.** Lord Agnew, thank you for providing to the Inquiry
25 a witness statement, INQ000536345.

1 It's signed. Can you confirm, please, that it's
2 true to the best of your knowledge and belief.
3 A. Yes.
4 Q. Thank you. By way of background and your role of
5 pandemic procurement, slightly unusually, I'm going to
6 just raise with you a number of comments that were made
7 of your role by somebody else, by Michael Gove.
8 Could we have, please, on the screen INQ000563560.
9 And at risk of embarrassing you, Lord Agnew, I'd
10 like to take you to certain parts of this statement.
11 The first is here at paragraph 9. It says this:
12 "Though Lord Agnew would report to me on broad
13 policy questions concerning commercial matters, I would,
14 in the main, leave the leadership to him as junior
15 minister for the Cabinet Office. I considered then, and
16 still consider now, that we were lucky to have an
17 individual of such ability, dedication and probity in
18 that role. During his time in Government he saved the
19 taxpayer millions of pounds and during the pandemic he
20 helped save lives."
21 Have you seen that part of Mr Gove's evidence?
22 A. No.
23 Q. All right. Well, I'll take you to two other parts and
24 then invite you to comment, if I may.
25 The second is at page 10 and paragraph 28, also

1 describing your role in pandemic procurement:
2 "Lord Agnew had a vitally important role within the
3 Cabinet Office [says Mr Gove], especially in these early
4 stages. Given his extensive commercial and Government
5 background, I wanted him to act as the lynchpin between
6 the Cabinet Office, [the] Treasury ... and [the
7 Department of Health and Social Care] to help with all
8 commercial elements of the pandemic, including
9 procurement. Lord Agnew agreed to undertake this role,
10 and I was kept abreast of developments in reports from
11 him, as and when required. Throughout those early
12 months of the pandemic, in particular, I recall
13 Lord Agnew went to enormous efforts, together with the
14 GCCO, to assist on many procurement issues with [the
15 Department of Health] and [the Treasury]."
16 Then finally, if I may --
17 A. Sorry, what's the GCC -- was that Gareth Rhys Williams?
18 Q. Yes, the commercial --
19 A. Right.
20 Q. Government.
21 Then, finally, at paragraph 106:
22 "Ultimately the effective discharge of public policy
23 during any crisis, such as a pandemic, depends on the
24 quality of individuals in office. I wish to thank the
25 superb team of ministers and officials with whom

1 I worked in the Cabinet Office who did a particularly
2 good job in very difficult circumstances, and I wish to
3 thank Lord Agnew most of all -- he is a hero."
4 So that is Mr Gove's description of your role. He
5 added, in oral evidence last week, that you devoted
6 a significant amount of your time to pandemic
7 procurement, albeit that he was unable to put an exact
8 percentage on it.
9 Do those descriptions of your role ring true with
10 you, Lord Agnew -- (overspeaking) --
11 A. You'll --
12 Q. Are they an accurate description --
13 A. You'll have to be the judge of that. I mean, I've never
14 been one for self-promotion.
15 I mean, in a way we were lucky because I'd worked
16 with him for nearly ten years before this whole crisis
17 blew up. I worked with him in the planning of the
18 education reforms in 2009. I was on the board of the
19 DfE when he came in as the Secretary of State. I became
20 the chairman of the academy's board, which was a one-man
21 board, me, to push through some of those policies.
22 I then went -- when he got sacked by Cameron, I went off
23 as his lead non-exec at the Ministry of Justice, which
24 is where I met Lady Hallett, actually.
25 Then he got sacked by May from there. And then,

1 very shortly after that, I became Education Minister in
2 the Lords, so, again, I did keep in touch with him. And
3 then he was the one who convinced me to take on this
4 benighted role after Boris won the election.
5 So, you know, we've been through a lot over the
6 years, but -- I mean, one of his strengths -- and I know
7 he can be a divisive figure for some people, but because
8 he has intellectual self-confidence, he's happy to
9 accept the things he doesn't know a lot about or have
10 particular expertise in. So he always brought around
11 him people to complement that.
12 Q. Lord Agnew, let me take you back to my question. There
13 was a reason for asking it.
14 A. Right. Sorry.
15 Q. I won't be the judge of it. Our chair will. But there
16 was a reason for asking you whether the description
17 given by Mr Gove of your role in pandemic procurement is
18 one that you regard as accurate. You give your own
19 description of your role in your own witness statement.
20 You tell us at paragraph 2 that on 14 February 2020, you
21 joined the Cabinet Office as a minister. Yes?
22 A. Yeah.
23 Q. And you say that by July of 2020 your focus was very
24 substantially redirected to a new role handled by the
25 Chancellor of the Exchequer, namely Her Majesty's --

1 HMRC's minister for Brexit Readiness?
2 A. Yes.
3 Q. Between joining the Cabinet Office and taking on the
4 HMRC role, you say that your involvement in
5 Covid-related procurement was focused on ventilator
6 procurement.
7 And you specifically make the point that you had in
8 your brief the designation "Procurement Minister", so
9 presumably that was more general than ventilator
10 procurement; that's a general procurement role. Yes?
11 A. Correct.
12 Q. Albeit you say this: the reality is that it accounted
13 for only about 5% of your workload.
14 A. Well, I was talking about my general brief as a minister
15 but obviously those first few weeks were utterly
16 chaotic, and I was heavily involved in procurement, but
17 very much around the ventilator programme because
18 I volunteered to be the minister that oversaw the work
19 that was done on that. In terms of the wider things, as
20 I said in my statement, one of the most useful things
21 I did was get Mr Gareth Rhys Williams to push a lot of
22 his good commercial people into the Health Department so
23 that they could get a grip.
24 But as you know from other statements I've made, the
25 Health Department did not welcome my involvement. And

1 so, as they started to get some momentum, I was closed
2 out of the process.
3 Q. I'm going to come up on to that in a few moments. What
4 I wanted to try to establish or resolve is an apparent
5 dissonance between the description you give of your own
6 role and the description given of your role by Mr Gove.
7 Do you see a dissonance between the two or not?
8 A. Well, I think you have to look at the different timings.
9 For example, if you fast forward a year or so, I was
10 heavily involved in the new Procurement Bill, which was
11 a post-Brexit piece of legislation, and that, a lot of
12 the learnings from Covid were put into the early
13 foundations of that Bill, and I was very heavily
14 involved with that.
15 Now, it's only just come into legislation, literally
16 I think a few weeks ago, so that took up a huge amount
17 of time. And then there was all the other procurement
18 stuff that I did, for example trying to get the Crown
19 Commercial Service to be much more assertive in its
20 provision of packages so that we weren't seeing all this
21 atomisation of purchasing across government, which of
22 course included some of this Covid stuff.
23 But I mean, if we go back to the Covid crisis on
24 procurement, you know, the real horror stories were in
25 those first six months. You know, the market across the

1 world very quickly stabilised, but that -- it was those
2 early days where it really was chaotic.
3 Q. Well, I'm going to come on to those six months now and
4 we're going to focus on your role within that time
5 period, and the appraisal that you give of the situation
6 that you observed. And you give quite a lot of comment,
7 concern, criticism, even, of the way that matters were
8 handled during that period. Is that a fair --
9 A. Yeah.
10 Q. -- summary?
11 A. Yeah.
12 Q. Can I just pick up one point before we delve into that.
13 You served as Minister for Brexit Readiness for the six
14 months preceding 1 January 2021; is that right?
15 A. It was probably a little bit less than that, but yeah.
16 Q. Are you able to say, relatively briefly, whether looking
17 back, preparations for Brexit helped or hindered the
18 public procurement challenge faced by the UK during the
19 pandemic?
20 A. Do you want to just repeat the question?
21 Q. Yes. Whether you're able to say one way or the other,
22 looking back, whether preparations for Brexit helped or
23 hindered the public procurement challenge which the UK
24 faced during the pandemic?
25 A. I mean, the problem is you were faced with two

1 simultaneous crises, really, because the Covid obviously
2 was the black swan. But on Brexit, as you will recall,
3 Cameron had very proudly told civil servants not to
4 prepare for a "no" vote. And then we were left in
5 a kind of limbo which took 3 years before Boris became
6 the Prime Minister and drove it through. I mean, I
7 don't know whether you're saying -- whether you're
8 leading me up to this question about the vaccines and
9 whether we --
10 Q. No, Lord Agnew. It's a neutral question. The Inquiry
11 has heard evidence in both directions.
12 A. Well, I mean, I think it was simple lack of bandwidth
13 across government. I mean the very fact that Gove was
14 responsible for much of the Covid crisis and then much
15 of the Brexit readiness implementation, so you had one
16 minister with -- under the most appalling pressure and
17 strain. So I -- I mean, I think there was one advantage
18 of Brexit -- of Covid that helped Brexit, which now I'm
19 just thinking aloud really. But on the D-Day of
20 1 January 2021, we didn't have the trade flows of
21 vehicles going through ports because there were still
22 lockdowns going on and all that kind of stuff.
23 So that did give us a bit of breathing space to test
24 all of the new -- I mean, we had 21 computer systems in
25 HMRC that had to be ready on that date.

1 **Q.** Lord Agnew, you've turned my question on its head.
2 You've found an example -- (overspeaking) --
3 **A.** Well, I have because I'm thinking aloud, and you're
4 opening up all the wounds that I've spent years trying
5 to forget.
6 **Q.** Well, let's try and keep it focused because we've got
7 limited time together this afternoon.
8 **A.** Okay, sorry. Well, there's my answer, that on the one
9 hand, there was lack of breadth, there was lack of
10 management breadth in government; on the other hand,
11 there was a tactical opportunity that did help us with
12 Brexit in those first few weeks because there were lower
13 trade flows of vehicles going through our physical
14 ports.
15 **LADY HALLETT:** One of the things I heard -- forgive my
16 interrupting -- in a previous module, was that
17 preparations for Brexit helped improve supply chains for
18 Covid. Do you have any comment on that?
19 **A.** Well, I can't think of any examples other than, you
20 know, there was talk of a vaccine which, because we were
21 leaving the EU, we had more confidence to go our own
22 way. I think technically we always had the ability to
23 go our own way but somehow there was a sense that, well,
24 actually, we're leaving anyway so let's do it ourselves.
25 But I can't -- I mean, I don't disagree with you, but

1 I can't think of any specific examples.
2 **LADY HALLETT:** Thank you.
3 **MR WALD:** Just to finish up on this, one of the themes in
4 your written evidence is that a lack of preparedness
5 created problems that resulted in, to use your word,
6 chaos, during those critical months. Was any of that,
7 in your view, lack of preparedness, down to the
8 diversion of attention onto preparations for Brexit?
9 **A.** No, because it should have gone back a lot further than
10 that. I mean, we hadn't had a civil contingencies
11 infrastructure in place in this country to my knowledge
12 for years. Not of any scale. So, no, I think that's
13 a red herring. I mean, it is a nice way for the
14 Remainers to blame Brexit, but I mean that's a total red
15 herring.
16 **Q.** All right, let's move on. You've got a section in your
17 written evidence on lack of strategy, and you say, in
18 fairly robust terms:
19 "There was no strategy or guidance. It was
20 firefighting."
21 **A.** Correct.
22 **Q.** Yeah.
23 **A.** Complete and total, which -- when we come to some of
24 these notes you've asked me to have ready, you know --
25 I was so -- I was worried about food. I mean, the

1 supermarkets were being stripped. I mean, you know,
2 everything was -- we were naked. Absolutely naked.
3 **Q.** Yes.
4 **A.** I think it's -- I want to stress that, because as I said
5 to you in the anteroom, everyone has forgotten that.
6 There's been a kind of national amnesia because "Thank
7 god it's all over, we can forget about it", but at the
8 time it was extremely frightening.
9 **Q.** Well, not everyone has forgotten that, but most of those
10 others were not in the same position as you were during
11 these critical months. They weren't Minister for
12 Procurement.
13 **A.** No.
14 **Q.** And what I want to understand is both the situation as
15 you found it but also what you did, what you tried to do
16 at the time to resolve those problems. And if you
17 failed -- as you say, in fairly stark terms, you did
18 fail -- what the reasons were for that failure, because
19 its important that lessons should be learned.
20 So let's just start with the lack of strategy. Is
21 that something you attempted to introduce? Did you do
22 your best to introduce strategy during your period of
23 involvement with public procurement?
24 **A.** It was too late. I mean, I only arrived in the
25 department in the middle of February. And so I was

1 confronted with a vast portfolio, with the unlikely
2 title of Minister for Efficiency and Transformation.
3 I mean, I found the list of jobs I was given. I mean,
4 there were 24 different roles, which I won't bore you
5 with reading them all out to you, but, I mean, the list
6 was simply ludicrous.
7 So the fact that -- did I start busily focusing on
8 civil contingencies as soon as I arrived in the DfE --
9 sorry, in the Cabinet Office and Treasury? No. I don't
10 even think I was the civil contingency minister,
11 actually, I think there was somebody else with that
12 role.
13 All I do remember, very clearly, which I put in my
14 witness statement, is this Sunday morning, two or
15 three weeks after I joined, when Gove summoned all his
16 ministers to Whitehall. And bear in mind I live in
17 Norfolk, it was a three-and-a-half-hour drive and so
18 I wasn't happy about it, but I hadn't understood the
19 severity of the crisis. And so I did drive from Norfolk
20 to Whitehall for that meeting.
21 He summoned various people, I think, including
22 Gareth Rhys Williams, and one or two senior procurement
23 people from the Health Department, and we were focusing
24 then on this total shortage of ventilators. I remember
25 that very clearly.

1 And the woman from the Health Department was saying,
2 "Well, we've got these orders from such-and-such
3 a factory in Europe, they promised they'd deliver them
4 last week, they haven't arrived."
5 I said, "Well, have you ordered a lorry to go and
6 get the damn things?"
7 You know, they were completely rabbits in the
8 headlights.
9 So we were into the crisis then, and that's when
10 I decided that where I could be most useful was where
11 I could take hold of a specific piece of the project,
12 which was the ventilators. Because that was outside the
13 Health Department, it had been announced as a Cabinet
14 Office initiative -- this was before I got involved --
15 and so I could see that I could run that, report to
16 Gove. It was a definable piece of work, which is why
17 I decided to put my effort behind that.
18 **Q.** Do you think it was the view of Mr Gove when he decided
19 to appoint you that it was too late in February for you
20 to do anything about the crisis? Do you think he shared
21 your view that you've expressed this afternoon, it being
22 too late?
23 **A.** I don't, I just honestly don't know. I think, you know,
24 the trouble is you get into a tumble dryer of events.
25 You're just thrown, literally, around in -- or in

1 a hamster wheel. And so we never had a strategic
2 conversation about events -- again, you have to sort
3 of -- I remember a most defining moment, when they
4 announced a lockdown in Wuhan, a city I'd never heard of
5 but I'd read in the paper it had a population of
6 10 million people, give or take. And I thought: this
7 will never happen in Britain or Europe, it's an
8 inconceivable event. You know, it was so far from our
9 reality.
10 Anyway, sure enough and then the problems arose in
11 Italy, and then -- and I remember sitting in the cabinet
12 room with Boris and a few other people and him saying
13 then, he said, "I'm being asked to restrict the freedoms
14 of British citizens by more than Churchill had to at the
15 worst stage of the Second World War."
16 I'll never forget that.
17 And I'm slightly labouring this, Mr Wald, because
18 it's so important that people can actually reverse their
19 memories back into these events of these horrific few
20 weeks and months.
21 So I don't know that there was clever planning --
22 and we always knew there was always this problem with
23 the Health Department and this ghastly quango -- which
24 they've now abolished, thank goodness. You know, it
25 had -- had a fortress mentality, and so people like

1 me -- and I'd only been there a few weeks, you know,
2 I wasn't -- they didn't know me and vice versa. So
3 I think that -- I think it's sort of -- I can't call
4 it -- there wasn't strategic thinking going on, no.
5 **Q.** Lord Agnew, I should mention, for the sake of the
6 stenographer, if you could try to keep your pace
7 a little slower --
8 **A.** I'm sorry, but it just brings back all these nightmares
9 of five years ago, almost to the day, actually.
10 **Q.** And for the sake of the Inquiry and the topics
11 that I was hoping to cover with you this afternoon, if
12 you could try to keep your answers -- I understand the
13 reasons for providing the evidence up until now that you
14 have been providing but it will help enormously if you
15 could try to keep your questions focused -- your answers
16 focused on the questions. Thank you.
17 You identify a particular problem when it comes to
18 strategy, inventory management, don't you?
19 **A.** (No audible answer).
20 **Q.** And in particular you say -- I don't think we need to
21 turn to it, but we can if it helps you -- at page 8 and
22 paragraph 37:
23 "I never saw any pre-pandemic modelling. But the
24 most egregious and easily fixed failure was a complete
25 lack of inventory management in hospitals."

1 And you say further:
2 "They had no clue what they held in inventory at any
3 level -- or at least it took a huge effort to get access
4 to this."
5 Then you mentioned that Number 10 brought in
6 Palantir.
7 Now, first of all, the problem of inventory
8 management, clearly that wasn't something that you could
9 have solved there and then, because it was
10 a pre-existing problem. Correct?
11 **A.** Correct.
12 **Q.** Did Palantir, a consultant, assist in any way?
13 **A.** Well, they were put in in Number 10. So they were
14 trying to bring some sense to the whole scenario. They
15 were trying to -- I mean, we -- I think, from memory, we
16 had something like -- maybe the numbers are wrong --
17 3,000 or 4,000 ICU beds in the whole country. I --
18 **Q.** Did Palantir assist in any way?
19 **A.** Well, exactly. So what they were trying to do was model
20 the journey of a hospitalised patient and how many --
21 what percentage of those patients were escalated from
22 a normal ward into an ICU, because the horror -- and
23 again, I wasn't at first, you know, in the front row on
24 this, but I was in the daily meetings for a few weeks --
25 the horror was that the standard wards would be

1 overwhelmed, they would be pushed into ICU and that ICU
2 wouldn't be able to cope. Palantir were trying to track
3 the disease, basically, and how it evolved.
4 So yes, I think -- but I'm not really the best
5 person on this, to be honest.
6 **Q.** The reason I asked you the question is that you later in
7 your statement express very negative views about the use
8 of consultants more generally. Is Palantir an exception
9 to that view?
10 **A.** Well, yes, it's a very different kind of beast. I mean,
11 Palantir are data gurus with an unrivalled expertise in
12 data mining and joining up of different systems to
13 produce data.
14 So what we had with the test and trace thing was
15 a grotesque jamboree, frankly. That's a completely
16 different issue -- (overspeaking) --
17 **Q.** At paragraph 46 you say:
18 "Everyone was flying blind."
19 This is still the problems in hospitals.
20 "For example, I tried for 3 months to get oxygen
21 capacity and consumption data for all hospitals as this
22 was a key determinant in how many ventilators a hospital
23 could support. I asked for a simple spreadsheet showing
24 oxygen delivery capacity per hospital. I never got it."
25 Why did you fail?

1 **A.** Well, I mean, I must have asked 20 times.
2 **Q.** Who did you ask?
3 **A.** Well, my private office were bombarding the health --
4 NHS England. I was speaking to the ministers over
5 there. As I say, I can't remember his name, I think it
6 was Edward Argar. They just didn't seem to understand
7 why this was so important.
8 Can I just explain why it's important? I know you
9 don't want long answers.
10 **Q.** If it's brief, yes.
11 **A.** Basically because there were two competing types of
12 ventilators, there was the CPAP and the invasive
13 ventilator. CPAP is a much easier, quicker solution for
14 patients, it doesn't need anaesthetists on hand, so
15 could be deployed much quicker and cheaper. The problem
16 is they're much more greedy on oxygen because you lose
17 a lot, you know, through the mask, whereas the tube is
18 going straight into your lungs.
19 So I wanted to know what the oxygen infrastructure
20 was. Even if we'd delivered hundreds of ventilators,
21 the intubation ones, it wouldn't have been any good at
22 all, as I've said in my statement, if they didn't have
23 the supply of oxygen to hook them up.
24 So the reason I gave up in the end is because
25 ventilators subsided as one of the critical solutions,

1 you know, we moved on, so I didn't persevere. It was an
2 example of the incompetence.
3 **Q.** Well, it is useful to, sort of, try and look at a higher
4 level, why that apparently relatively simple request
5 resulted in failure, essentially?
6 **A.** Well, it's a very good question. I mean, what I did,
7 I did give myself a crash course in oxygen and
8 ventilators from a very good person who installed oxygen
9 delivery systems to hospitals. And in a 20-minute
10 little teach-in Saturday morning, he explained. So all
11 the lights came on for me. It was very quick to
12 understand. Maybe these other people just didn't have
13 that insight to understand why it was important.
14 I mean, I had another issue which I was worrying
15 about -- I don't want to talk too much, but this was
16 about where you diverted ambulances to with patients who
17 needed intubation. There was no point sending them off
18 to a hospital that only had, say, ten ventilators, and
19 I wanted to see a grid of hospitals, numbers of
20 ventilators and oxygen supporting those ventilators.
21 A cascade of stuff. I didn't achieve anything along
22 those lines.
23 **Q.** All right. I want to turn to a topic which I know is of
24 very, very considerable concern to you. Spending
25 controls.

1 **A.** Mm.
2 **Q.** In fact, it was in relation to spending controls that
3 you eventually resigned from government; is that right?
4 **A.** Well, I mean, yes, I suppose so. It was the -- it was
5 the fraud that really got to me. But it all connects,
6 to be honest with you.
7 **Q.** All right. Well, let's turn to that topic now. One of
8 your roles was Minister for Spend Controls, wasn't it?
9 **A.** Correct.
10 **Q.** What did that involve? What did it mean?
11 **A.** Well, when I arrived I inherited a very demoralised team
12 and the first submission I got was "We'd like to shut
13 down spend controls", and given I'd been put in as
14 minister for efficiency, I wasn't going to get done by
15 that. I mean, that's a standard procedure in
16 government. They'll hit a new minister with something
17 they'd like to slip past him or her quickly before
18 they've found their feet in their new role. But
19 I wasn't having that so I pushed back and we kept it,
20 and then gradually I built the morale of the team up.
21 To give you an idea of the volume of money in the
22 year of Covid that came across our desks, it was in
23 excess of £140 billion.
24 **Q.** 140?
25 **A.** Billion.

1 **Q.** Billion. You say at your paragraph 31:
2 "... I discovered that several departments had
3 obtained exemptions from this process."
4 **A.** Correct.
5 **Q.** "The biggest of the [Department of Health] followed by
6 the Ministry of Defence. From memory, the value of the
7 [Department of Health] exemption was a shocking
8 £70 billion/year of spend that should have come through
9 this scrutiny but didn't have to."
10 **A.** Correct.
11 **Q.** Now, I just want to pause there before I move on to the
12 second half of this paragraph. How does one strike the
13 necessary balance, in a crisis, of the time that is
14 necessary to scrutinise spend, and the urgent need for
15 spend in a volatile market?
16 **A.** Well, if I told you that one of my subs over that time
17 was for £11 billion, which I finally got reduced to
18 about 600 million, it gives you an idea of the sort of
19 sloppy thinking going on around government. So --
20 **Q.** Let's come back to my question then-- (overspeaking) --
21 **A.** -- (overspeaking) -- of course there's a natural
22 tension, but one of the things I prided in my private
23 office, we turned things round very, very quickly. You
24 will never -- you won't find an official who had any
25 dealings with my private office who will say, "Agnew was

1 dragging his feet, he was slow in replying to
2 submissions."
3 I agreed service standards. I said as long as we
4 get X number of days, you will get dealt with.
5 So this idea -- this is the sort of lazy sloppy talk
6 that you get, "Oh well, we're saving the world, we can't
7 be bothered with him, we're going to spend the money as
8 we like", and you've got this in the pack, this one
9 about the tests, *(redacted)* quid a test, one and a
10 quarter billion, and I was given a day to approve it.
11 I mean, honestly-- (overspeaking) --
12 **Q.** -- (overspeaking) -- I'm going to come on to that.
13 **A.** Yeah.
14 **Q.** I'm coming on to that in a moment. The one and a
15 quarter billion and the 24-hour date for approval, but I
16 want to just stay with this paragraph. You say:
17 "I fought hard to remove these exemptions with some
18 success, but it took over 18 months. But the limitation
19 of this role was that by the time submissions to spend
20 money reached me, the horse had pretty much left the
21 stable and it was extremely difficult to amend decisions
22 that the civil service machine regarded as done and
23 dusted."
24 **A.** Correct.
25 **Q.** So the 18 months, did your position last 18 months?

1 **A.** Yeah, I was there for just a fraction under two years.
2 **Q.** And you said you had limited success within that time?
3 **A.** Mm-hm.
4 **Q.** What was the effect of that delay, that amount of delay,
5 of 18 months?
6 **A.** Well, I don't know -- because what I didn't see
7 I couldn't tell you about. But we did reduce the amount
8 of exemptions. It was derailed on the Health Department
9 because just as I had finally got Hancock to agree to
10 it, he got fired. And so we had to start all over again
11 with the next chap. So -- but we did achieve something.
12 But I bet if you looked at it now, they would have
13 slithered their way out of it and whatever exemptions
14 I managed to remove would have been reinstated --
15 because that is how it works. If you do unpalatable
16 things to the machine, the machine bites back,
17 ultimately.
18 **Q.** You make specific criticisms of the experience you had
19 in relation to spending controls in your paragraphs 61
20 to 65. I just want to go through them quickly:
21 "Based on the outcomes I do not think there was
22 effective and timely coordination between [daily
23 procurement meetings] and [the Department of Health]
24 about procurement spending."
25 What do you mean by that?

1 **A.** Well, I think it was -- that they were so panicked that
2 they just went rushing off to get whatever they could.
3 And so there was no coordination that I could see.
4 **Q.** You then said:
5 "The [Department of Health] seemed to adopt
6 a 'whatever it takes' approach. I have already
7 explained what limited impact I was able to have."
8 Now, we heard earlier in this module from Steve
9 Barclay --
10 **A.** Mm.
11 **Q.** -- who made a similar -- who expressed a similar
12 concern, particularly in relation to the limited time
13 that he and his colleagues had to scrutinise requests
14 for spending.
15 **A.** Correct.
16 **Q.** Was it your view that those requests could and should
17 have come sooner than they did?
18 **A.** Absolutely.
19 **Q.** To what do you attribute the lateness of the receipt of
20 those requests?
21 **A.** Well, two reasons. One, simple sloppiness; and second,
22 the basic they could just roll me over because they
23 could pull this "It's a national emergency" card, you
24 know, "Stop, get out of our way".
25 **Q.** And how do you know it is the latter rather than --

1 **A.** Well, I don't know it's the latter but I'm just working
2 on human nature, frankly, and if you go back to
3 Steve Barclay, who, again, for the record, is one of the
4 most impressive ministers I ever worked with, and I was
5 lucky to have him as the Chief Secretary when I was in
6 my role because we dovetailed on a lot of the things.
7 I mean, you saw that example on the billion and
8 a quarter. I wasn't consulted nor was he consulted.
9 You know, let's just spend the money.
10 **Q.** Well, let's just turn to that. I said I would. Let's
11 turn to it now.
12 It's INQ000471020.
13 And this is the email exchange.
14 **A.** You see, even our permanent secretary was -- his
15 normally very calm demeanour was stirred by this.
16 **Q.** There it is-- (overspeaking) --
17 **A.** There we are, paragraph 5.
18 **Q.** It's point 5 there:
19 "I would like to understand why, when we have known
20 for at least 3 ½ months that testing is a vital plank in
21 dealing with this disease, I was given less than one day
22 to approve [you say] [more than a £1 billion]
23 transaction."
24 It was 1.25 billion, wasn't it?
25 **A.** Yeah.

1 **Q.** So what did you have to do, in the circumstances?
2 **A.** Well, I fought my normal rearguard action, which
3 I think, in here, or over the page, are various
4 conditions that I applied or wanted to apply.
5 I think I even said: if you don't do these things
6 then you'll have to write a formal letter of direction
7 to -- which then at least forces accountability and
8 responsibility back to the people trying to spend the
9 money in this extremely undisciplined way.
10 And eventually, Alex Chisholm, who was a very -- we
11 had a good relationship, and he was always calming, you
12 know, putting oil on troubled waters, he came up with
13 a deal there, which was -- hardly satisfactory, but,
14 given the pressure we were put under and the timescales,
15 that's what we had to end up with.
16 **Q.** All right. And I think at page 3 you make a similar
17 remark about how ridiculous it was to ask for that
18 approval?
19 **A.** Yeah. Well, what do you think? I mean, you know, tell
20 me. I mean, you know, if you think I'm mad, tell me.
21 **Q.** Not my role --
22 **A.** Okay.
23 **Q.** -- Lord Agnew, to provide that view.
24 Let's turn --
25 **A.** You know, the country has to get angry about this sort

1 of incompetence. That's the point, Mr Wald. And that's
2 why I get so upset about it. People think it's all
3 being done in a marvellously organised way, and it's
4 not. It certainly wasn't then.
5 **Q.** Let's turn to another document. It's an excerpt from
6 Chris Hall's second statement.
7 It's INQ000536421, paragraph 28:
8 "There was a real need" it starts.
9 "Will acknowledged" --
10 **A.** So we are in a very different context here now, aren't
11 we? This is the early stages of trying to secure masks
12 and things-- (overspeaking) --
13 **Q.** This is exemptions, exempting PPE from spending
14 controls.
15 **A.** Oh, right.
16 "Will acknowledged that there was a need to propose
17 an alternative approach to Lord Agnew, Minister of
18 State, and suggested either making PPE exempt from
19 spending controls or providing a blanket approval
20 subject to certain conditions. I endorsed the latter
21 approach and requested that Will consult with Gareth
22 Rhys Williams, the Chief Commercial Officer, before
23 approaching Lord Agnew."
24 Of those two, which do you think was preferable?
25 **A.** Well, I'd lost confidence in their ability to strike

1 reasonable deals. So that's why I didn't have -- didn't
2 agree with either of them. I think I was overruled in
3 the end. I can't remember now, to be honest. But ...
4 **Q.** All right. Let's turn on, as I said I would, to the
5 subject of private consultants. You clearly had
6 concerns about the role and value of consultants in the
7 government procurement effort?
8 **A.** Mm.
9 **Q.** Is that right?
10 **A.** Correct.
11 **Q.** Yeah.
12 **A.** I mean, it wasn't all bad. We had -- the people in the
13 ventilator team were good, PA Consulting, they were
14 a good team, but it was a small team and it was very
15 tightly managed.
16 **Q.** You say that you were appalled at the costs of the
17 consultants?
18 **A.** Yeah, I mean, again, just for context, government was
19 already addicted to consultants. As I put in my witness
20 statement, they spent -- Whitehall spent 1.8 billion on
21 consultants before Covid. So this was an addiction --
22 it wasn't a new addiction, it was simply building on the
23 heroin addiction they already had. So, you know, it
24 wasn't anything new. And it was something I could see
25 very soon after I arrived was extremely bad practice.

1 **Q.** That's 1.8 billion over which timeframe?
2 **A.** I can't remember. I think it was -- it was the
3 financial year before Covid. So Covid was -- maybe the
4 financial year ending April 2019. Something like that.
5 **Q.** You say at your paragraph 33:
6 "I had limited interaction with him other than being
7 appalled at the costs [of consultants]. I tried to
8 reduce this, in particular with Deloitte and the call
9 centre they established in [the Department of Health]
10 for Baroness Harding where they had over 15,000 employed
11 with a FTE utilisation rate of less than 5%."
12 **A.** Yes, let's just break that up quickly. So there weren't
13 15,000 consultants but there were about a thousand
14 Deloitte consultants and about 100 of their partners,
15 and, between them, they cooked up a distributed call
16 centre of around 15,000 people. So -- just so you
17 understand it.
18 But the idea that you needed a call centre of
19 15,000 people with a utilisation rate of 5% is beyond
20 a joke. I mean, you understand what I mean by
21 utilisation rate in a call centre? So that's the
22 number --
23 **Q.** I do. It's total billable hours over total hours
24 available.
25 **A.** Exactly. Now, they were rightly concerned that you

1 couldn't have the call centre overwhelmed because of the
2 nature of -- the highly sensitive nature of trying to
3 track and trace people, but 5% was a joke. And so
4 that's why I tried to get the 15,000 reduced. I think
5 we got it down to about 11,000 briefly, but as soon as
6 my back was turned it went back up again.
7 **Q.** Lord Agnew, you were Procurement Minister, Minister for
8 Spend Controls; you had a role within government for
9 improving efficiency?
10 **A.** Correct.
11 **Q.** What did you do about all of this?
12 **A.** Well, I think that when Baroness Harding had got the
13 role as the test and trace tsar, she had got herself an
14 exemption from oversight of a lot of this by us. So
15 I was irrelevant. So I -- you know, I would make a lot
16 of noise because I felt so strongly about it, but
17 ultimately the levers of power did not exist. As you
18 will, I'm sure, have heard the expression, in
19 government, you pull a lever, and there's nothing
20 connected. And that was an example.
21 **Q.** You would say that the -- because we've now encountered
22 a number of examples where your attempts to improve
23 matters failed. You would say that was entirely
24 a feature of the circumstances that you faced, rather
25 than your approach to it?

1 **A.** Ha! Well, you may think I was too blunt, too direct,
2 too aggressive. Fair criticisms, I think. But these
3 were problems that existed before Covid. I think it's
4 important to understand that. And I think for most
5 people who are interested in the machinery of
6 government, there is a widespread recognition that the
7 system is broken in many places. So -- but if I was --
8 if I lacked the diplomatic skills to bring people on
9 board, then that's a perfectly fair criticism. I'm not
10 known for being a soft-soaper.
11 **Q.** I suppose the reason for the question, Lord Agnew, is
12 that you identify within your written evidence, at
13 paragraph 11, page 3, two interventions. In fact you
14 describe them as your only two significant
15 interventions.
16 One of them you touched on a few moments ago, and
17 that was encouraging Gareth Rhys Williams, the
18 government commercial officer, to lend up to
19 50 procurement staff at the Department of Health to help
20 them accelerate procurement. And you made that
21 suggestion in a meeting in the Cabinet Office called by
22 Michael Gove on the first or second Sunday of
23 March 2020. Yes?
24 **A.** Correct.
25 **Q.** And that was actioned?

1 **A.** It was actioned, and -- (overspeaking) --
2 **Q.** And it resulted in an improvement?
3 **A.** I think so. I hope so.
4 **Q.** Yes.
5 **A.** And actually, as Gareth told me subsequently, that
6 number was -- increased by a factor of fivefold or even
7 more. So yes, I think it --
8 **Q.** I think we touched on those figures when Sir Gareth gave
9 evidence to this Inquiry.
10 **A.** Yes.
11 **Q.** Then you identify a second significant intervention for
12 which you were responsible. You said:
13 "I obtained funding from The Treasury to buy several
14 mask manufacturing machines in an attempt to create some
15 local manufacturing capacity. These were imported from
16 China but by the time we got them up and running, world
17 supply chains had created a flow of masks that were much
18 cheaper. The idea was to try to create some capacity
19 for non NHS needs."
20 **A.** Correct.
21 **Q.** So that was a second significant intervention that was
22 effective?
23 **A.** Well, not really, because the market adjusted, and so
24 the -- you know, I mean, it was a -- it was effective in
25 what it set out to do, but the market caught up with us,

1 and so the price of masks came down to a level that
2 these relatively small machines could not compete with.
3 So I -- you know, I don't want to make a big thing
4 of this being a huge intervention, but I was very
5 worried about places like schools, you know,
6 bus drivers, you know, all these areas that weren't
7 getting the masks initially and all the priority was
8 clearly going to the hospitals. But I'm not -- it
9 wasn't a game changer.
10 **Q.** So I come back to the reason for my question. We're
11 five years on from many of these events now.
12 **A.** Mm.
13 **Q.** There has been an opportunity for reflection and
14 introspection. Everyone will remember the events of
15 those five years ago. Very few were in a position, as
16 you were, to be able to influence how the crisis was
17 handled. Do you think, on reflection, that you might
18 have contributed more than one or two significant
19 interventions?
20 **A.** Well, I certainly saved quite a lot of money. I mean,
21 that 150-odd billion I mentioned on the spend controls,
22 you know, there were a lot of savings if you aggregated
23 them over the period of time. So I earned my keep in
24 that area.
25 But when I wrote that, I was talking more from

1 a top-down perspective of what -- we spent £350 billion,
2 give or take. We have left a huge debt legacy to our
3 children's generation, who will have to struggle to pay
4 it off, given all their other problems, and the country
5 has not recovered its productivity. We have damaged
6 a whole generation of children -- I see that from my
7 work in schools -- who were put wrongly into lockdowns,
8 given all of that.
9 So if I look across the wreckage of all of that, I'm
10 pretty horrified. Tactically, yes, I was -- you could
11 say I was a platoon commander defending the perimeter of
12 Dunkirk. It helped get those 200,000 soldiers off the
13 beach but it was a military disaster in its own right,
14 you know.
15 So, you know, I think it's important that we
16 calibrate where I am putting myself, you know, here.
17 **Q.** Lord Agnew, was one of your other interventions creating
18 the VIP Lane?
19 **A.** No, I'm not -- I mean, I was pushing for clarity on what
20 was happening because we were forwarding these offers
21 into the system. And certainly I said, you know, "Can't
22 we have some clarity about this?" Because I was being
23 chased by people and then, you know, again, every
24 evening on the news there'd be talks to such-and-such
25 a hospital running out of gowns or nursing homes with no

1 masks. I mean, again, you have to remember the
2 environment we were operating in.
3 **Q.** You say in your statement that you were not responsible
4 for creating the High Priority Lane or the VIP Lane?
5 **A.** I don't think I was. I mean, I pushed for -- I saw one
6 of your papers, there was somebody -- I think it was
7 a civil servant that suggested we have some sort of --
8 I mean, you have to have some sort of order, don't you?
9 **Q.** Let's just have a look at what you say and what others
10 say and then I'll invite your view on that.
11 Can we have INQ000536345. It's paragraphs 16 to 17
12 at page 4:
13 "I have been asked to describe my understanding of
14 how the idea for and establishment of the High Priority
15 Lane came about. I have always been puzzled by this.
16 The only role I played was to talk to potential
17 suppliers who contacted either myself or Michael Gove to
18 try to establish whether they were credible. If
19 I thought they were, I would refer them to the
20 Government commercial team."
21 And then at the paragraph below you say:
22 "I did not have a role in the establishment,
23 operation or supervision of the HPL other than referring
24 credible sounding people to the CS commercial team."
25 Could we now turn to the witness statement of

1 Clare Gibbs, corporate witness statement from the
2 Cabinet Office.
3 It's INQ000528389, at page 54, paragraph 3.82.
4 Clare Gibbs tell us:
5 "On 24 March 2020, Andy Wood, the PPE Buy Cell
6 leader sent a readout of a virtual meeting with
7 Lord Agnew, Minister of State at the Cabinet Office ..."
8 **A.** Well, what's wrong with that? I mean, a fast track --
9 **Q.** Hang on, hand on:
10 "... and HM Treasury which had taken place the
11 previous day. Lord Agnew had asked if a 'fast track'
12 could be set up to process 'ministerial/seniors emails
13 regarding offers of help that they were sending
14 through'."
15 **A.** Isn't that a logical thing to do?
16 **Q.** Isn't that the VIP Lane?
17 **A.** Well, I don't know. As I said in my affidavit, my thing
18 a minute ago, I've been puzzled by this. I mean, surely
19 you'd have thought we were mad if there wasn't some
20 prioritisation.
21 **Q.** Well, that's a separate point, Lord Agnew. It may be
22 a logical thing to do, it may be a sensible thing to do.
23 I'm just trying to establish whether it was your
24 brainchild.
25 **A.** Well, if it was then so be it. But we had to have some

1 logic to it. I mean, I don't ... if I'm the godparent
2 of this thing, maternity nurse, so be it. I mean, you
3 seem to have some sort of an obsession with this, from
4 just listening to your interrogation of other people.
5 It was absolutely vital that we had some order. And if
6 you look at, in fact, the papers that you've just
7 provided just when I got here earlier today, the
8 percentage of contracts that went through this -- well,
9 you've called it an HPL, I'm not quite sure which is
10 which, but I presume it's the same -- you know, they
11 were a smaller number of contracts for a larger sum of
12 money which made it viable.
13 Yet you, and you were advocating that in your
14 interrogation of Gove the other day. So I'm, frankly,
15 very confused at why people are in such a state about
16 this.
17 **Q.** You think it was a good idea?
18 **A.** Well, we had no alternative.
19 **Q.** Would you reinvent it, faced with similar circumstances?
20 **A.** I think faced with the circumstances of having -- being
21 naked as a country with all these supplies, and that
22 there were credible people contacting us with offers of
23 help, yes, I would.
24 **Q.** Let's just, to complete this, display INQ000498337,
25 page 2.

1 This the readout to which I've just referred in
2 which Andy Wood says this:
3 "Another action I took [towards the bottom] was to
4 advise Lord Agnew if we could fast track
5 ministerial/seniors emails regarding offers of help that
6 they were sending through. We developed a public
7 *proforma* ..."
8 And so on.
9 Is that consistent with what you remember?
10 **A.** I mean, I don't disagree with what he's written, but
11 I just -- I mean, I really struggle to understand why
12 this is such a point of obsession. You know, we really
13 had nothing, and so if there were credible people out
14 there who could help, then we should have gone with
15 them. I mean, it was that simple. I think the
16 mistakes, with hindsight, as I think I put in my
17 statement, were we kept using what are called -- what
18 are they called, single source -- I can't remember the
19 term now, but basically we should have re-tendered after
20 the first initial crisis had passed, and there wasn't
21 enough of that.
22 So single sourcing. I think that went on too long.
23 But I don't apologise for creating some order in the
24 system, if I was responsible for it. I mean, I think
25 it's quite funny that suddenly where I failed to bring

1 any kind of impact everywhere else, I'm now considered
2 the creator of this thing which has been so
3 controversial. I do regret when it was -- we had this
4 fellow, this lawyer fellow taking a case against us
5 later that year. I regret that I didn't stand up and
6 say all that then. But by then I was so embroiled in
7 HMRC Brexit readiness, I was barely aware of what was
8 going on-- (overspeaking) --
9 **Q.** You're referring to the Good Law Project?
10 **A.** Yeah. I mean, I --
11 **Q.** There were two cases, weren't there?
12 **A.** Yeah --
13 **Q.** There were two cases?
14 **A.** But I was sort of -- I was so diverted by then that I
15 wasn't -- it was barely on my radar. But I do regret
16 not standing up publicly to defend what we did and I'm
17 absolutely unapologetic. Yes, we got plenty of stuff
18 wrong. Yes, probably some crooks came through the
19 VIP Lane. Yes, we've got this Baroness Mone^ woman,
20 who perhaps you're going to ask me about in a minute --
21 **Q.** No, I'm not.
22 **A.** Well, that's a relief, because the NCA will deal with
23 her. But I'm sure we got stuff wrong, Mr Wald. But,
24 you know -- I mean, let me give you an example with this
25 Inquiry. The first witness statement I get, it's

1 riddled with duplications. About a third of the
2 questions I got were duplicates. And this is a machine
3 with an unlimited budget moving at a glacial pace.
4 **Q.** Lord Agnew, you've raised the Good Law Project
5 litigation --
6 **LADY HALLETT:** Just before you go on, can I interrupt, just
7 because of certain comments made in the press about the
8 Inquiry.
9 Lord Agnew, when Counsel for the Inquiry ask
10 questions, it doesn't indicate an obsession, it doesn't
11 indicate a set view. It certainly doesn't indicate any
12 view on my part. The questions are designed just to
13 explore an area of public concern, in some areas of the
14 public, and also to give people like you a chance to
15 answer, just as you are.
16 **A.** That's fair enough, Lady Hallett, I think -- I mean, but
17 I listened to Gareth when you were grilling him on this
18 and he carried out two inquiries with GI something or
19 other, some commercial unit, taking what he described as
20 potentially the most toxic cases. You know, he took it
21 very seriously when these concerns were raised. You
22 didn't seem in any way reassured by what he said, but it
23 sounded credible to me. But, I mean, I wasn't involved
24 in any of that by this stage.
25 But I really do think this obsession with the High

1 Priority Lane is misconceived given the circumstances
2 that we were faced with.
3 I mean, I want to give you -- I know I'm talking too
4 much but I want to put this analogy. In 1940,
5 Lord Beaverbrook was made Armaments Minister, he got
6 hold of the Castle Bromwich Spitfire factory and from
7 making no Spitfires by the end of that year they had
8 built 350 Spitfires. We lost 326 in the Battle of
9 Britain, which was in around the same time.
10 Now, in your world, you wouldn't have had a rogue
11 like Lord Beaverbrook doing something like that. He was
12 a philanderer, he was a bully, he was an egotist. But
13 if he hadn't done what he'd done, we would possibly have
14 lost that battle, which would have cleared the way for
15 an invasion.
16 Sometimes you have to have odd people in the system.
17 And that's all we were trying to do, which is -- no
18 doubt we'll come on to that with Sir James Dyson and the
19 ventilators. You seem terribly upset about his role.
20 The bloke is a genius --
21 **LADY HALLETT:** Again, you're attributing comments or --
22 **A.** Well, I'm only listening, Lady Hallett, to what I've
23 heard with-- (overspeaking) -- poor old Gove got grilled
24 for 20 minutes on Dyson.
25 **LADY HALLETT:** You did work for the Ministry of Justice,

1 Lord Agnew. Basically when counsel ask questions, it
2 doesn't indicate any settled view on their part --
3 **A.** Okay, fair enough.
4 **LADY HALLETT:** -- or any conclusions they've reached and,
5 I repeat, that I have reached. So please don't think
6 that we have reached any conclusions.
7 **A.** All right. But I want to try and reassure you, but I am
8 not being very successful, that there was not some
9 conspiracy going on here. We were trying to react in
10 real time to the biggest crisis in this country for
11 70-odd years. That is the reality of it. We got plenty
12 wrong. And I've said repeatedly in my statement, I feel
13 overall that my contributions were pretty limited
14 because look at the results, 350 billion quid, hundreds
15 of thousands of children damaged by lockdown,
16 productivity unrecovered -- it's not a good outcome, is
17 it?
18 **MR WALD:** Lord Agnew, I'm grateful for your comments on the
19 VIP Lane, not least because the contrast that they offer
20 with those comments that have been provided by previous
21 witnesses. I don't think a previous witness has said
22 that it would be reinvented in that form. For one
23 reason or another, previous witnesses have concluded
24 that there were problems with it such that it should not
25 be reinvented faced with a crisis of similar type. But

1 you would say yes, one would need to reinvent that
2 system in those circumstances.
3 **A.** If we were as unprepared next time as we were last time.
4 **Q.** All right. Let me move on and I do, with some
5 hesitation, because I suspect this is a topic that may
6 result in a further increase of the blood pressure.
7 It's excess PPE and waste, I'm afraid, which is
8 another topic about which I know you feel very strongly.
9 You express your view on the success or otherwise of
10 PPE procurement at paragraph 45 of your statement, and
11 you say this:
12 "... if one steps back from the fray and sees the
13 colossal amount spent on PPE, much of which is now being
14 burnt, one can hardly regard it as a success."
15 **A.** Correct.
16 **Q.** Now, am I right that this topic also fell within the
17 various remits that you held as minister within the
18 relevant period?
19 **A.** Well, I think, when I got word of it, it just wound me
20 up so much that I had to use whatever influence I had to
21 try to do something about it. I can't -- the honest
22 answer is I can't remember. I think at one stage maybe
23 somebody asked me to deal with all of the chaos in the
24 docks, because, as I mentioned, Felixstowe dock at one
25 stage had 10,000 PPE containers piling up inside it,

1 and --
2 **Q.** Yes, you say so at paragraph 96 of your statement.
3 **A.** Yeah.
4 **Q.** You say:
5 "Disposal of surplus PPE was one of the most
6 upsetting parts of the whole procurement journey. There
7 was vast over-ordering. At one point it almost blocked
8 Felixstowe docks. Penalty rates were being paid on
9 seaborne containers that were kept because there were no
10 storage facilities. Warehouses were rented eventually
11 but then began the destruction of PPE."
12 And I know that Mr Marron, at the beginning of his
13 evidence, was taken to the scale of waste. It was an
14 enormous scale of waste. I don't think I need to go
15 back to the figures, but he agreed that in July, the end
16 of July of 2021, there were 6.9 billion items of surplus
17 PPE. That was a figure that reduced down to 4.9 billion
18 by 2023, but it continued to cost £120 million a year to
19 store excess PPE and £3.8 billion worth of stock expired
20 before it could be used.
21 So these are enormous sums.
22 **A.** Yeah. Very upsetting, but again, I'm conscious of not
23 talking too much.
24 All I would say to you, the lesson to be learned and
25 I just hope you put this front and centre when you do

1 put your report in, is that if there had been inventory
2 management in the hospitals, this need never have
3 happened. And inventory management is not complicated.
4 Every chain of clothing stores in the country uses it.
5 I would use the example of Next.
6 If Lady Hallett goes into Next in Croydon to buy
7 a dress, and they don't have the colour she wants, they
8 will say, "We've got your colour in your size in
9 Oxford Street. You can go there or if you wait two days
10 and come back again, we will have it brought to the
11 store in Croydon."
12 That is all inventory management is. It is that
13 simple. And that is what it should be in every one of
14 the 1,200 hospitals. And if that had happened, one, at
15 the beginning, if they'd had that, which why on earth
16 one of the most, allegedly, advanced economies in the
17 world not have that embedded into its health system is
18 beyond my comprehension. But that should be the lesson
19 we've learnt. Has anything happened in the 5 years
20 since Covid? I bet it hasn't.
21 **Q.** All right.
22 Lord Agnew, you anticipated that I would ask you
23 about Dyson --
24 **A.** Indeed.
25 **LADY HALLETT:** Sorry, just before you go on.

1 Are there any other ways, Lord Agnew, in which you
2 think the excess, the waste, could have been avoided.
3 I mean, the team that were trying to get the PPE were
4 under huge pressure, at that stage they had no idea how
5 many more waves there were going to be, what the burn
6 rate would be, and, as you say, they didn't have an
7 inventory management system. Are there any other things
8 you think could have been put in place to prevent the
9 excess buying and the waste?
10 **A.** Well, when I got involved, I did try very hard, because
11 there was still -- Covid was still a worldwide issue,
12 and there were lots of countries, particularly
13 Commonwealth countries that needed equipment and
14 support, and so I tried to use that. But I just got
15 lost in the quagmire of the Foreign Office, DFID, Health
16 Department, all these people. They wouldn't work
17 together as a team and, you know, even donate this stuff
18 to --
19 **LADY HALLETT:** But that's distributing the excess.
20 **A.** Exactly.
21 **LADY HALLETT:** My question really was: is there anything
22 else that could have been done, do you think, to prevent
23 that excess arising, or was it inevitable, given the
24 circumstances, that the teams faced? Were there any
25 other ways of controlling the expenditure or reducing --

1 **A.** Well, I was -- I mean, with hindsight, I think, you
2 know, we had the success of the vaccine, which did
3 rather kill the disease. So we didn't know that, so we
4 over-ordered, which -- but it was an over-ordering of an
5 order of magnitude far greater than needed to be because
6 we didn't have proper data to show, as I mentioned
7 earlier, to show the journey from -- you know, I got the
8 Covid. I didn't know I had it until three months later.
9 I was very lucky, I only got a very mild burst of it.
10 So we didn't know about the level of severity that would
11 hit different people.
12 So it's -- I would urge you to focus on inventory
13 management, actually.
14 **LADY HALLETT:** Yes, thank you.
15 **MR WALD:** Thank you, my Lady.
16 If I can just supplement that question with one
17 further that arises out of it. Do you say that, given
18 problems with inventory management, the scale of the
19 over-ordering was inevitable, or was nonetheless
20 excessive? There are witnesses that have said they
21 would rather be sitting where you sit and explaining
22 over-ordering than under-ordering.
23 **A.** Yeah, well, I agree with that.
24 It was a lot of the stuff around the size of it,
25 though, Mr Wald. So this whole storage issue was such

1 a catastrophe. So we were paying huge surcharges on
2 seagoing containers because they hadn't organised proper
3 storage facilities. So, I mean, to give you some idea,
4 a seagoing container costs about, I don't know, four or
5 five times a day what an ordinary one that just sits in
6 a static -- you know, they hadn't got any of that
7 organised.
8 I said to them, "Harness the farming community."
9 I said, "You know, as a farmer myself, we've got huge
10 sheds all around the country", and if you're in, say,
11 the onion business, they have to be temperature
12 controlled, humidity controlled. There were all sorts
13 of opportunities. I couldn't get anyone to take any
14 notice of me.
15 **Q.** But what I'm keen to understand in relation to this is
16 whether the scale of over-ordering, the scale of waste,
17 the eventual £3.8 billion worth of stock expired,
18 unusable, at the end, is the, in your view, inevitable
19 consequence of inventory management problems, or is
20 excessive, even taking into account inventory management
21 problems?
22 **A.** I think it's excessive. I mean, I accept that we should
23 have been on the side of caution, and ordered a bit more
24 than we thought we needed. But my frustration is that
25 we hadn't a clue what we had when we were ordering more.

1 **Q.** Which seems to suggest that it's almost random, how much
2 you end up with. If you simply don't know what you've
3 got or when the vaccine might be rolled out, or when
4 other imponderables might eventuate, then you just keep
5 ordering. Is that the position?
6 **A.** Well, that's what seemed to happen, yeah.
7 **MR WALD:** All right.
8 My Lady, I wish going to turn to another topic.
9 I don't know whether you're minded to take an early
10 break.
11 **LADY HALLETT:** I wasn't, unless you want me to.
12 **MR WALD:** Okay, then I'll keep going.
13 Let's start off with what you say about ventilator
14 procurement. We'll go back to your -- I'll just read
15 from your statement, if I may, at page 3, paragraph12.
16 You say:
17 "I played a bigger role here as I volunteered to be
18 the minister overseeing efforts to manufacture them in
19 the UK. Michael Gove appointed me to this oversight
20 role in March 2020 just after a competition had been
21 announced for UK suppliers to come forward with designs
22 and plans."
23 Yeah?
24 **A.** Correct.
25 **Q.** So we then -- we know from the witness evidence of

1 Mr Dan Webster that you were the primary Cabinet Office
2 minister involved in the Ventilator Challenge, and that
3 you made all relevant ministerial decisions. Yes?
4 **A.** Yeah.
5 **Q.** You were involved in daily meetings with Gareth Rhys
6 Williams and others in the Ventilator Challenge from
7 22 March 2020 until 20 April of the same year, when the
8 frequency of those meetings was reduced.
9 **A.** Yeah, we met six nights a week, including a Sunday, and
10 importantly, given your concerns about Dyson and
11 everything else, the MHRA were there I think virtually
12 every evening. It was a truly collaborative effort.
13 **Q.** We heard from Sir Gareth about the Technical Design
14 Authority within the Ventilator Challenge programme. It
15 was the mechanism, was it not, by which it was possible
16 to assess the progress of designs against the
17 specifications that the MHRA insisted on.
18 **A.** Mm-hm.
19 **Q.** And, more specifically, whether a design was likely to
20 meet the technical specification and medical need?
21 **A.** Correct.
22 **Q.** And it included senior representatives from the MHRA, as
23 did your meetings, senior clinicians, and the Medical
24 Devices Testing and Evaluation Centre?
25 **A.** Mm-hm.

1 **Q.** We know from Mr Webster that the Technical Design
2 Authority made recommendations, which were then
3 considered by yourself, who would decide which projects
4 to continue and which to stop.
5 **A.** Mm-hm.
6 **Q.** The purpose of the Technical Design Authority was to
7 make recommendations, ie to yourself, who was the
8 ultimate decision-maker, about what to stop and what
9 continue, based principally on the assessment of likely
10 medical utility.
11 **A.** Correct.
12 **Q.** And then, finally, Mr Webster says that the primary
13 criterion for the selection of designs in the Ventilator
14 Challenge was clinical efficacy.
15 **A.** Correct.
16 **Q.** Which has its own meaning in the context of
17 a ventilator.
18 **A.** Well, you know, I took -- I took a lot of advice on this
19 stuff because I didn't know what a ventilator looked
20 like when I started, so ...
21 My job really was to proffer leadership and momentum
22 and clarity of mission and to support all of the people
23 involved, to give them the empowerment that they needed
24 to get on with it as fast as possible.
25 **Q.** So could we have INQ000512992. And we'll start at the

1 bottom and work our way up, I think.
2 **A.** Can you expand it a bit?
3 **Q.** Yes.
4 **A.** Sorry.
5 **Q.** Can you read that?
6 **A.** No.
7 That's better. Thank you.
8 **Q.** There you are.
9 If we could start with Sir Gareth's email and then
10 look at your response.
11 "Dear Lord Agnew,
12 "Apologies for the lack of a formal sub ..."
13 Anyway, the second paragraph, he says that the --
14 what had come to his attention was a reduction in the
15 demand for ventilators. You may remember this.
16 **A.** What was the date of this one? 10 April.
17 **Q.** 10 April.
18 "Until Wednesday evening you were encouraging us to
19 make as much as possible, recognising that many of the
20 40,000 firm orders plus the LOIs were not going to
21 actually be achievable due to product or manufacturing
22 failure, et cetera, but we now recommend that we
23 throttle back considerably."
24 Do you remember that --
25 **A.** I mean, I don't remember the chronology of events, but

1 I do remember this. And as I said, it's back to my
2 obsession with oxygen supplies as well. I think maybe
3 there was a dawning realisation that we simply
4 couldn't -- the system could never plug in anything like
5 this number of ventilators.
6 **Q.** Well, there was a dawning realisation here that the
7 numbers of irons in the fire was too great.
8 **A.** Right, yeah.
9 **Q.** And in fact, Sir Gareth, towards the end of this email,
10 he says:
11 "The implication is that:
12 "- we should kill off ..."
13 A number of different options:
14 "... Remora
15 "- we should kill off the Helix ...
16 "- we should allow those products going to the test
17 house this weekend to continue ... The list of likely
18 candidates to be struck off are ...
19 "- OxVent
20 "- Florence
21 "- Dyson
22 "- Mosquito
23 "- Jarrehead and
24 "Apollo 13
25 "In that order."

1 So Dyson is third on the list.
2 "Early next week ..."
3 Then just towards the end of this email, we can
4 finish on there -- a bit further down, please. There we
5 are.
6 "The key decisions we will need from you and other
7 ministers, are whether you feel those 18,000 and 30,000
8 targets are appropriate, and if you want us to
9 manufacture (how many more) for export or DFID use. We
10 can then trim the previous volume asks to match. I hope
11 this makes sense!"
12 And he says before that:
13 "I appreciate this is quite a lot to take in so
14 I wanted to flag this with you as soon as possible so we
15 could discuss on Monday."
16 Presumably Sir Gareth there, he recognises that this
17 email contains quite a significant change of gear, or
18 a recommendation for that.
19 **A.** Mm.
20 **Q.** Yeah?
21 **A.** And what did I say?
22 **Q.** Let's see what you said.
23 If we go to the top of this email:
24 "Gareth,
25 "Many thanks for your note.

1 "I pretty much agree with all of it. By killing off
2 the 'no hope' designs you free up resources to really
3 concentrate on getting the viable ones through
4 regulatory testing and onto the battle field.
5 "A couple of points
6 "a. I'm v keen that we try to salvage ..."
7 The key one is (c):
8 "We are going to have to handle Dyson carefully.
9 I accept that contractually we can walk away as he
10 hasn't delivered by the due date."
11 Do you remember that? He didn't deliver by the due
12 date?
13 **A.** Well, I don't think anybody had. But, I mean, I don't
14 disagree with it. It's a statement of fact. I don't
15 remember the micro -- you know, micro-series of events.
16 **Q.** Okay.
17 "I also accept that we have an indemnity battle
18 ahead."
19 Do you remember what you meant by that?
20 **A.** Well, we had -- this was a problem, actually, with all
21 of them, I think, which is why, even at the end, we kept
22 several in play, because a couple of them hard-balled us
23 on indemnity -- you know, basically said, "We want
24 a blanket indemnity from the government. If our
25 machines kill anybody, you know, we're out, we're not on

1 the hook." And that was something that worried me very
2 much.
3 **Q.** Okay, but you seemed to give Dyson's design possessions
4 treatment here. You say:
5 "But just killing off his design (assuming it gets
6 through MHRA) won't be an option. I suspect we'll have
7 to buy a few machines, get them into hospitals so that
8 he can [then] market [them] internationally being able
9 to say they are being used in UK hospitals."
10 **A.** Assuming it gets through the MHRA, by the way.
11 **Q.** You make that very clear --
12 **A.** I mean, I know you got in a state about this in
13 an earlier --
14 **Q.** Don't worry about Mr Gove's evidence.
15 **A.** Well -- well, only that clearly you were worried that
16 another one of these heinous plots that --
17 **Q.** Lord Agnew, you make as plain as a pikestaff that this
18 is --
19 **A.** Okay.
20 **Q.** -- contingent on MHRA approval.
21 **A.** Good, okay.
22 **Q.** It's there for everyone to see.
23 **A.** I don't want the narrative going off into the --
24 **Q.** No, no --
25 **A.** You know, that I'm a Bond villain --

1 **Q.** That was last week's evidence, we're focused on
2 -- (overspeaking) --
3 **A.** -- trying to put -- (overspeaking) -- machines out
4 there.
5 **Q.** "I suspect [you say] we'll have to buy a few machines,
6 get them into hospitals so that he can then market
7 internationally being able to say that they are being
8 used in UK hospitals."
9 **A.** Yeah. Well, at that stage, I assumed he would produce
10 something that worked. I mean, he was -- he threw
11 a hell of a lot of money at this, as you will know, and
12 I mean, I -- he was sending us daily updates, he used to
13 call them night letters, where he had teams in
14 Singapore, I think, and also in his Malmesbury factory.
15 You know, he was throwing a level of R&D at it that
16 dwarfed anybody else. So I think it's important to
17 remember that.
18 I know you think he's been given some favouritism
19 here, it may be that we have, but the rate of which he
20 was pushing his own development gave me that
21 reassurance.
22 **Q.** If he were able to produce a ventilator that worked, why
23 would you only be showcasing it in a few UK hospitals in
24 order that it could be exported aboard?
25 **A.** Well, I was looking for a way to save face with him, if

1 he produced a viable product. But we were also
2 conscious that the design advice that Gareth put up in
3 his note was that he felt that there would be better
4 models than that. But again, at that stage we really
5 didn't know. We didn't know what the rate of
6 manufacturing capacity was of these people and nor did
7 we know about this indemnity issue which I mentioned
8 a few moments ago.
9 So I don't think it was a mistake to keep Dyson in
10 play until we got to the point at which we knew his
11 machines couldn't work.
12 **Q.** It was your job to let Sir James Dyson know that --
13 well, to -- (overspeaking) --
14 **A.** It's the hardest phone call I have ever had to make in
15 my professional life.
16 **Q.** Hardest phone call you've ever had to make in your
17 professional life?
18 **A.** Yeah.
19 **Q.** Why was that?
20 **A.** Because this was a bloke who was rung up by the
21 Prime Minister and said, "We've got a real problem here.
22 We know that you're one of the most successful
23 manufacturing entrepreneurs in this country, please help
24 us."
25 He took -- he took up the flag and he threw

1 everything at it. And he made an enormous amount of
2 effort over those preceding few weeks or maybe months.
3 And so, you know, when the day came that we -- it didn't
4 work or that it didn't -- you know, MHRA said, "Sorry
5 this isn't a goer", you know, I was in charge of this
6 thing, I was the one who had to ring him and tell him.
7 And this is a man who is not used to failing. I mean,
8 how many people do you know in this country who have
9 made £20 billion from a standing start? You know, you
10 don't get to that level of success by tolerating
11 failure.
12 So it was a very difficult call and luckily we both
13 have a Norfolk connection, and I was able to build on
14 that connection, and I was able to deliver the news to
15 him, I hope in a civilised way and to recognise the huge
16 commitment -- I mean, they say he spent 20 million quid
17 of his own money. How many other people did that during
18 Covid for this country?
19 **Q.** Lord Agnew, we have dealt with some lessons learned
20 through the course of your oral evidence. I wanted to,
21 and indeed you deal with a lack of lessons learnt within
22 your reason evidence. Your own view is that lessons
23 have not been learnt, isn't it?
24 **A.** Correct.
25 **LADY HALLETT:** Are we moving on to lessons learned?

1 **MR WALD:** Yes.
2 **LADY HALLETT:** In which case I think we'll take the break
3 now, Mr Wald.
4 **MR WALD:** Well, I don't think there's anything that remains
5 because --
6 **LADY HALLETT:** Oh right, sorry.
7 **MR WALD:** It may be that -- is there anything you wish to
8 add in terms of the lack of lessons learned or lessons
9 that should be learned beyond that which you have said
10 in your oral evidence?
11 **A.** No, I mean we talked about the inventory management. On
12 these consultants I did achieve the creation of an
13 internal government consultancy service which was
14 starting to reduce the dependency on external
15 consultants, but that was abolished after I left.
16 So this is the problem. I mean, if you look at Kate
17 Bingham, I'm sure you'll have her in here on some point,
18 if you haven't already on the vaccines.
19 **Q.** Already been in.
20 **A.** I'm sure -- she's done YouTube lectures on it how the
21 machine just reverted to type within weeks of her
22 leaving. All that wonderful leadership and energy she
23 brought. The DNA doesn't take into the system. This is
24 the bit I just don't understand. They don't see the
25 lessons and say, "That's how we'll do it from now on."

1 It's always revert back to the same, plodding
2 mediocrity.
3 **Q.** And if you went into that role --
4 **A.** God help me.
5 **Q.** -- in similar circumstances -- excuse me?
6 **A.** God help me that I would ever do that.
7 **Q.** Is there anything you would do differently?
8 **A.** No, because unless I had the absolute backing of
9 a prime minister that I could do things that would mark
10 your hair curl, I wouldn't do it.
11 **MR WALD:** Lord Agnew, thank you. Those are all my
12 questions.
13 **THE WITNESS:** Thank you.
14 So what happens -- sorry.
15 **LADY HALLETT:** Well, basically we can either take a break or
16 we can move to Ms Morris's questions. You've got about
17 20 minutes, Ms Morris, is that right?
18 Are you happy to carry on?
19 **THE WITNESS:** Yeah.
20 **LADY HALLETT:** Or would you rather we took a break.
21 **THE WITNESS:** I mean, as long as you're happy, let's carry
22 on.
23 **LADY HALLETT:** Ms Morris?
24 **Questions by MS MORRIS KC**
25 **MS MORRIS:** Thank you, my Lady.

1 Lord Agnew, I ask questions on behalf of Covid
2 Bereaved Families for Justice UK.
3 **A.** You are going to have to speak up a little bit, I'm
4 slightly deaf.
5 **Q.** I represent over 7,000 bereaved people who lost their
6 loved ones during the Covid pandemic. It's a UK-wide
7 group that represents probably just one section of the
8 public that does have concerns about the existence of
9 the VIP Lane, and so I will be asking you some more
10 questions about it, whether that makes me obsessed or
11 otherwise, I hope you'll understand it's an important
12 area that the Covid bereaved would like some answers to.
13 You've said today that you don't understand sort of
14 public concern about the VIP Lane. I'm going to ask you
15 to look at, please, a section from Sir Christopher
16 Wormald's statement, and that's INQ000571763 at
17 paragraph 43, please.
18 **A.** Can you expand it? Thanks.
19 **Q.** I'm very grateful. It says here:
20 "I became aware of a 'high priority referral' route
21 when others referred to it, but I was not involved in
22 concluding that such a route was needed, nor do
23 I remember being asked whether it should be created. My
24 view is that the principle of triaging referrals and
25 giving priority to the most promising was correct. The

1 problems that arose were in how the prioritisation took
2 place, with too great an emphasis placed on who made the
3 referral, as opposed to the nature and promise of the
4 lead. The criteria used to assess the proposal was the
5 same for all matters of referral; the problem was not
6 with a differing standards of assessment, but with who
7 got to the front of the queue. And as a result, some
8 less promising leads were given too great a priority."
9 Do you accept that view, Lord Agnew?
10 **A.** No, I mean, that's a perfectly reasonable statement, and
11 I'm not going to disagree with it. But again, I just
12 want to go back to where we were as a country at the
13 early stages. I want to reassure the people you
14 represent. You know, we didn't have any mask
15 manufacturing capacity in this country. We had one
16 factory, which was the 3M factory, but they're an
17 American-owned factory and the American government could
18 have deployed what they call the Defense Act which
19 requires any American-owned corporation anywhere in the
20 world to send their production back to America. We had
21 nothing. So you must get that through to your people --
22 **Q.** So your point, as you said to the chair, is that because
23 of how poorly prepared we were, there was no other
24 choice-- (overspeaking) --
25 **A.** Yes. I mean --

1 **Q.** All right.
2 **A.** -- exactly. With hindsight, of course there's wonderful
3 things you could have done --
4 **Q.** I understand.
5 **A.** -- but we didn't have hindsight.
6 **Q.** I understand that.
7 **A.** And I want to reassure your people that you act for,
8 there was no heinous plan to enrich a few of our mates.
9 I mean, that's such bollocks, if you forgive my
10 language. You know, we were in the most terrible
11 position, and this was -- they were credible people who
12 came forward, mostly. There was, no doubt, one or two
13 crooks and cranks but they were largely credible and
14 it's borne out in the volume of stuff they ended up
15 deleting.
16 So I just want you to get that through to your
17 people. This was not some kind of plan, right wing
18 people trying to enrich themselves.
19 **Q.** Well, it will be the chair to make those decisions and
20 to forgive any language that's used in her courtroom.
21 **A.** I apologise. But it's important because they've been
22 whipped up, your people, by a narrative which has not
23 been properly countered. And that's why I feel so
24 strongly about this. And I blame myself. At times, as
25 say it to Mr Wald, when this court case was brought,

1 I was too distracted to not stand up then and make more
2 noise about it.
3 **Q.** I understand. And you'll understand that the questions
4 that I put are on behalf of individual people, members
5 of the public, and those who have lost their loved ones.
6 So if we can just focus on the questions that I ask
7 rather than comments on how they may or may not feel
8 about it, I would be very grateful.
9 **A.** No, but the point is this was done to save those people.
10 **Q.** I understand. I understand the evidence. Thank you.
11 You note at your paragraph 68 of the statement
12 you've provided to the Inquiry that the counter fraud
13 function of the Cabinet Office, was not properly
14 utilised, in your view, by the Department of Health and
15 Social Care.
16 So did that include in the PPE Buy Cell? Is this an
17 area where you think the counter fraud function should
18 have been more looking for those "crooks and cranks", as
19 you say?
20 **A.** Well, I can't speak with any specific knowledge because,
21 as I say, they operated their kind of walled approach to
22 us, but --
23 **Q.** Okay --
24 **A.** And I'm not familiar with how much fraud there was in
25 procurement at the end of it, so -- because that would

1 have taken a while to manifest itself.
2 But I don't think that they were -- given the volume
3 of transactions, they should have leant on us more
4 heavily.
5 **Q.** And how could they have helped in preventing fraud in
6 the procurement?
7 **A.** Well, because we had people in the centre, in the
8 Cabinet Office, who were real international experts.
9 I mean, the second-most senior person in our
10 counter fraud team in the Cabinet Office was actually --
11 we sold a day a week of his time to the Australian
12 Government, he was that highly regarded. So there was
13 real expertise there and I don't think it was used
14 enough, but I can't give you specific examples.
15 **Q.** Thank you.
16 Moving on now to my final topic, please. The Good
17 Law Project identified that Uniserve Limited,
18 a logistics company, was awarded contracts for both
19 logistics and for PPE valued at over £800 million in the
20 early stages of the pandemic. Their contracts were
21 worth over 300 million alone, however, it's publicly
22 available that 182 million items that were supplied by
23 the company, valued at 178 million, were deemed unusable
24 by the NHS.
25 You have been identified by the DHSC as the High

1 Priority Lane source of referral for Uniserve's offer to
2 supply PPE. Was Iain Liddell somebody that you knew
3 personally, or through political connections or
4 otherwise?
5 **A.** No, I mean, I don't even know what Uniserve is. What is
6 it? What is it?
7 **Q.** Were you aware that they had no track record in the
8 supply of PPE?
9 **A.** No, but we used a lot of people who didn't because we
10 had to go wherever we could. But I had no relationship
11 with them.
12 **Q.** Okay, so if that's correct, on what basis did you
13 consider their offers merited urgent or priority
14 consideration within the VIP Lane?
15 **A.** I can't remember.
16 **MS MORRIS:** Thank you.
17 Thank you, my Lady, those are my questions.
18 **LADY HALLETT:** Thank you very much indeed, Ms Morris.
19 That completes the questions we have for you,
20 Lord Agnew. I hope we haven't opened too many wounds.
21 I can tell there are very high levels of passion in some
22 of your responses. But I promise you, we are
23 investigating everything thoroughly. We're trying to
24 give people the opportunity to answer the questions that
25 have caused people concern. So thank you for the help

1 that you've given to the Inquiry.
2 **THE WITNESS:** Thank you.
3 **LADY HALLETT:** Very well.
4 10.00 tomorrow; is that right?
5 **(3.09 pm)**
6 **(The hearing adjourned until 10.00 am the following day)**
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